NCE EXAM PREP

Practice Exam 03

170 Practice Questions

225 minutes | A, B, C answer choices

Questions followed by answer key and detailed explanations

> **Instructions.** Select the single best answer for every question. Record your answers separately. Work at exam pace, allowing approximately 225 minutes for all 170 questions. The answer key and explanations begin in Part B; avoid viewing them until you have completed Part A.

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PART A — QUESTIONS

Choose one answer (A, B, or C) for each question.

Question 1

> A high school senior receiving school-based counseling for attention and organization difficulties will attend college in another city. The student wants to use campus counseling and disability supports but has never requested accommodations independently.

Which transition activity best promotes continuity and self-advocacy?

A) Ask the parent to arrange all college services so that support is in place before classes begin

B) Help the student identify documentation and access steps, practice making contact, and confirm a backup plan

C) Wait until academic problems occur because college services should respond only to demonstrated failure

Question 2

> A 12-year-old in foster care wants help maintaining a homework and calming routine. A placement change is possible, and the current foster parent participates in planning. The child already uses a portable notebook successfully at school.

Which plan is most likely to remain useful if the placement changes?

A) Base the routine entirely on the current household's rules and reward system

B) Build portable notebook cues and child-owned coping steps, with caregiver supports that can be explained to a new placement

C) Delay routine planning until the child's long-term placement has been finalized

Question 3

> The counselor uses the term “cognitive distortion.” The client looks confused and says, “I don’t know what that means, but I’m embarrassed to keep asking.”

What is the counselor’s best response?

A) Apologize for the jargon, explain the idea in plain language with the client’s example, and check whether it makes sense.

B) Give the client a glossary to review after the session so counseling time remains focused.

C) Continue using the term consistently because repeated exposure will make it familiar.

Question 4

Which finding most directly indicates that a planned counseling approach should be deferred or modified?

A) The client expects the approach to require effort and temporary discomfort

B) The client asks to begin at a slower pace than the counselor usually recommends

C) Current conditions make the approach likely to cause harm or require abilities the client does not yet have

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Question 5

> A 17-year-old in a youth community program will turn 18 and transfer to the agency's adult service next month. The client wants a parent involved only in appointment reminders and is ready to make other care decisions independently. Agency policy requires new adult consent and releases at age 18.

Which transition plan best follows the client's preferences and the stated policy?

A) Continue the youth-service permissions automatically until the parent requests a change

B) Exclude the parent from all communication because adult services never involve family members

C) Complete the adult consent process, document the limited reminder role, and confirm the first adult-service appointment

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Question 6

> A prelicensed counselor has frozen during suicide assessments and omitted important follow-up questions in two sessions. Worried that disclosure will affect an evaluation, the counselor considers seeking advice from an outside consultant without informing the clinical supervisor.

Which professional development response is most appropriate?

A) Use only the outside consultant so the counselor can correct the problem before the next evaluation

B) Continue practicing independently and disclose the omissions only if a client later requires emergency care

C) Bring the performance concern to the clinical supervisor and develop a monitored improvement plan, using consultation only as a supplement

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Question 7

> A clinic is considering generative artificial intelligence to translate psychoeducational and consent materials into several languages. Staff members have not studied the reliability or professional implications of machine-generated clinical translations.

Which learning step should occur before the clinic relies on the translated materials?

A) Obtain independent education on translation errors, cultural and linguistic bias, confidentiality, human review, and appropriate disclosure

B) Learn how to shorten prompts because concise instructions consistently produce accurate clinical translations

C) Use the translations immediately and ask bilingual clients to report any errors they notice

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Question 8

> Assessment by an integrated care team identifies a client's first episode of psychosis. The client wants to remain in school and agrees to referral to a local coordinated specialty care program that provides counseling, medication services, family education, and supported education.

What is the best continuity step for the current counselor?

A) End counseling after giving the program's telephone number because specialty care now owns the case

B) Refer only for medication because educational goals should remain outside the specialty program

C) Facilitate the program intake and maintain an agreed bridge until the receiving team confirms care

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Question 9

> A client has just returned home after an emergency mental health evaluation and says the discharge plan “turned into a blur.” The client has the written plan, is currently stable, and wants help understanding what to do if distress rises again.

Which intervention is most appropriate?

A) Review the plan in short sections, connect each step to a likely situation, and ask the client to explain how the steps would be used.

B) Replace the clinical plan with a simpler plan created solely by the counselor.

C) Ask the client to reread the plan alone because changing its presentation could create confusion.

Question 10

A strong correlation between two assessment variables indicates which conclusion?

A) The variables are related, but the correlation alone does not establish that one causes the other

B) One variable necessarily causes changes in the other

C) The two variables measure exactly the same construct

Question 11

> A client misses an appointment. The insurer does not reimburse missed sessions, while the signed financial agreement permits a reasonable client cancellation fee when notice is insufficient.

How should the practice handle the missed appointment?

A) Submit a therapy-service claim because the counselor reserved the clinical hour

B) Bill both the insurer and the client so the practice can recover the full scheduled fee

C) Apply the disclosed cancellation policy to the client without billing the insurer for a service not delivered

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Question 12

> An active client is president of a nonprofit and invites the counselor to join its board. Board members evaluate proposals the client submits, and the counselor is drawn to the prestige and community influence the position would offer.

How should the counselor respond?

A) Accept the invitation but abstain only when the client's proposals come to a vote

B) Examine the attraction to the role, obtain consultation, and decline the overlapping position while counseling is active

C) Accept the position after telling the client that board discussions will remain separate from counseling

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Question 13

> After making a minor mistake during a presentation, a client feels intense shame and wants to cancel a valued community event. The client agrees that withdrawing would likely deepen the shame and wants to act differently.

Which intervention best fits the situation?

A) Encourage the client to wait until the shame disappears before deciding about the event.

B) Help the client plan a manageable form of opposite action, such as attending briefly and engaging with one supportive person.

C) Ask the client to argue that the presentation mistake was actually a success.

Question 14

> A clinic computer displays a ransom note, and users can no longer open the electronic health record. The incident plan directs staff to isolate affected systems, preserve evidence, notify the security team, and activate approved downtime procedures.

What should the counselor do?

A) Follow the incident and downtime procedures immediately without attempting an unauthorized repair or payment

B) Keep entering information into the affected computer so delayed notes do not accumulate

C) Pay the requested amount personally because rapid access is more important than incident review

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Question 15

> A client corrects the counselor's pronunciation of the client's name for the third time and says, "I'm starting to think getting it right doesn't matter to you."

Which response best supports alliance repair?

A) "I work with many unfamiliar names, so I hope you can understand that mistakes happen."

B) "You have corrected me more than once, and I still got it wrong. I'm sorry. Please help me say it correctly now, and tell me how this has affected you."

C) "I do care, so I'll write the pronunciation in my notes and make sure we move forward."

Question 16

> A client requests an accounting of disclosures. The applicable rule and clinic procedure require a response within 60 days and identify which nonroutine disclosures must be included and which treatment, payment, and operations disclosures are excluded.

What should the counselor or clinic do?

A) Route the request through the required process and provide the accounting within the deadline

B) Give the client the treatment record because it is equivalent to an accounting of disclosures

C) Refuse the request because disclosure logs are internal compliance documents only

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Question 17

> After nearly every comment in group, a member looks at the leader and asks, "Was that okay?" Other members have begun waiting for the leader's judgment instead of responding to one another.

What is the leader's best intervention?

A) Reassure the member after each comment until enough confidence develops.

B) Stop responding to the member's questions so the behavior will extinguish.

C) Describe the approval-seeking pattern and invite the member and group to explore what happens when the leader does not evaluate each contribution.

Question 18

Which statement best distinguishes reflective practice from unproductive rumination?

A) Reflective practice eliminates uncomfortable emotion, whereas rumination always increases it

B) Reflective practice examines experience in a structured way to inform future action, whereas rumination repeats distress without useful movement

C) Reflective practice is completed alone, whereas rumination occurs only when discussing concerns with others

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Question 19

> Following a frightening assault, a client sometimes feels detached from the body and says the surroundings seem unreal. During these episodes, the client knows the sensations are internal experiences and does not believe the world has literally changed. Substance and medical causes have not yet been assessed.

Which interpretation best guides further assessment?

A) Preserved reality testing rules out any clinically significant dissociative symptoms

B) The description suggests depersonalization or derealization phenomena that require assessment of trauma, impairment, duration, and possible external causes

C) The report establishes a delusional disorder because the client describes the environment as unreal

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Question 20

When selecting a norm-referenced instrument, why is the composition of its norm group important?

A) A large norm group guarantees that every individual score is valid

B) Norms eliminate the need to consider the client's language and educational experiences

C) Score comparisons are more meaningful when the norm group is relevant to the population and purpose of use

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Question 21

> A client with chronic pain entered counseling to improve sleep and participation in family activities. After eight weeks, pain ratings are unchanged, but the client sleeps an additional hour most nights, attends family dinners again, and describes life as more manageable.

How should the counselor interpret these findings at the progress review?

A) Treatment is ineffective because the primary physical symptom has not decreased

B) Treatment is complete because improved functioning makes further review unnecessary

C) Treatment is producing meaningful functional gains, and the client should help decide whether the remaining plan needs revision

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Question 22

> A high school student is safe after peers circulated a humiliating altered image online. Notifications keep arriving, and the student says, “Every alert makes it start all over.” The student wants help regaining control without losing possible evidence.

Which intervention is most appropriate?

A) Tell the student to delete every account immediately so the harassment can no longer be seen.

B) Ask the student to keep checking the posts so the full scope of the harassment is understood.

C) Help the student silence alerts, preserve necessary evidence with a chosen supportive adult, and decide how and when to access the accounts.

Question 23

A client previously gave a spouse proxy access to the client portal. During a separation, the client revokes that permission and reports concern that the spouse may read new counseling messages. The system can end proxy access immediately.

What should the clinic do?

A) Wait until the next monthly access review so the change follows the normal schedule

B) Revoke proxy access promptly, document the change, and review any related privacy or safety concern

C) Keep access active until the spouse also agrees to the revocation

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Question 24

A client reports occasionally hearing the client's name called while falling asleep, then immediately realizes no one spoke. The experience occurs only at sleep onset, causes no impairment, and is not accompanied by daytime hallucinations, disorganization, mood episodes, or substance use.

What is the best assessment interpretation?

A) Consider a sleep-transition phenomenon and continue monitoring context rather than diagnosing a psychotic disorder from this isolated experience

B) Diagnose schizophrenia because any auditory experience is sufficient evidence of psychosis

C) Diagnose dissociative identity disorder because the voice uses the client's name

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Question 25

During a mental status examination, a client describes receiving a major award but shows little facial expression, minimal vocal variation, and almost no observable emotional response. The client reports feeling pleased internally.

Which description best characterizes the observed affect?

A) Labile

B) Euphoric

C) Blunted

Question 26

A caregiver misses medical appointments because the available bus route requires three transfers. The caregiver says, “I keep calling myself irresponsible, but I need a plan that actually works.”

Which counseling intervention is most appropriate?

A) Challenge the word “irresponsible” until the caregiver reports less guilt.

B) Encourage stronger commitment to the existing route before considering alternatives.

C) Define the transportation problem precisely, brainstorm options without judging them, and evaluate one feasible next action.

Question 27

Which practice best describes an effective counseling paraphrase?

A) Repeating the client's exact words so the original meaning cannot be altered.

B) Restating the essence in concise language and giving the client an opportunity to confirm or correct it.

C) Adding a likely motive that the client has not yet expressed.

Question 28

Which action best demonstrates a closed-loop referral?

A) Give the client a current list of several agencies and wait for the client to mention the referral again

B) Select one agency for the client and assume that sending its contact information completes the referral

C) Confirm whether the client connected with the selected service and, with the client's involvement, address barriers or identify an alternative

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Question 29

> A bilingual counselor wants to translate an English depression scale for Somali-speaking clients. The counselor is clinically fluent in both languages but has no training in test translation, cultural adaptation, or validation.

What is the most appropriate assessment decision?

A) Translate the items literally because language fluency preserves the scale's psychometric properties

B) Translate the scale and compare scores only among clients seen at the counselor's agency

C) Locate a supported version or consult qualified assessment and cultural-language experts before using translated scores

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Question 30

> A supervisor discovers that a supervisee documented and billed three counseling sessions that did not occur. Agency policy requires preservation of records, immediate removal of billing access, investigation, correction, and reports to affected parties or regulators when required.

What should the supervisor do?

A) Ask the supervisee to repay the claims privately and keep the matter out of the personnel file

B) Wait for a payer audit because supervisors are not responsible for a supervisee's billing entries

C) Activate the required corrective process, protect clients and records, and make any mandated reports

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Question 31

A residential program offers a private practitioner $100 for every client referred and asks the counselor not to mention the payment. Several other programs provide comparable services without referral payments.

What should the counselor do?

A) Accept the payment if each referred client ultimately enrolls in the program

B) Split the payment with the client so both parties benefit from the referral

C) Decline the undisclosed referral fee and base recommendations on client needs and service quality

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Question 32

To save time, a counselor copies last month's progress note into a current encounter. Before signing, the counselor sees that the copied text incorrectly states the client denied substance use and attended with a spouse.

What should the counselor do?

A) Sign the note because copied language is acceptable when the treatment goal is unchanged

B) Delete the entire encounter and document nothing because the draft contains errors

C) Replace the inaccurate text with current, individualized documentation and review whether similar errors entered prior notes

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Question 33

Which approach provides the strongest foundation for a counseling advocacy initiative?

A) Announce the counselor's preferred solution before gathering information so decision-makers hear a clear message

B) Engage affected people, define the barrier with relevant data, and identify feasible targets and partners for change

C) Begin public criticism of the responsible organization before checking whether the reported barrier is widespread

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Question 34

A worker witnessed a serious workplace accident in which a colleague was injured. Several hours later, the worker is safe but shaking and saying, “I should have prevented it.” Emergency personnel have confirmed that the worker did not cause the accident.

What is the counselor’s best response at this stage?

A) Help the worker settle physically, validate the distress, and identify an immediate supportive person or practical need.

B) Challenge the guilt by reviewing every fact proving the worker was not responsible.

C) Ask for a detailed sensory account of the accident so the memory can be processed before the worker leaves.

Question 35

> Two family members request counseling together. One wants reconciliation after a long estrangement, while the other wants only help setting boundaries and has no intention of restoring the relationship. Each assumes the counselor will support that person's goal.

What should the counselor do before beginning conjoint services?

A) Adopt the reconciliation goal because restoring relationships is the purpose of family counseling

B) Accept both clients without discussing the incompatible expectations until conflict arises in session

C) Clarify each person's goals and the counselor's role, then determine whether there is a mutually understood and suitable basis for conjoint work

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Question 36

> A client says, “Since my promotion, my spirit feels crowded.” The counselor is unfamiliar with the phrase and has several possible interpretations.

What is the counselor’s best response?

A) Reflect that the client feels anxious because the promotion created too many demands.

B) Avoid the phrase and ask only about observable changes since the promotion.

C) Ask the client what “my spirit feels crowded” means in the client’s own experience.

Question 37

Which statement best describes professional mentoring in counselor development?

A) Mentoring can support career growth and professional identity but does not replace required clinical supervision or establish competence by itself

B) Mentoring transfers responsibility for the counselor's clinical decisions to a more experienced colleague

C) Mentoring is equivalent to supervision whenever the mentor and counselor discuss client work

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Question 38

> A school district plans to assign counselors primarily to test scheduling and routine disciplinary monitoring. Counselors have data showing that their counseling and prevention activities are associated with improved attendance and service access.

What is the most appropriate professional advocacy response?

A) Refuse all testing and disciplinary duties before discussing the plan with district leaders

B) Use role standards, local outcome data, and stakeholder input to propose duties aligned with students' counseling needs

C) Accept the reassignment because administrators alone determine the appropriate professional role of school counselors

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Question 39

An elementary-school client uses a picture-based emotion scale in counseling. With the child and parent's authorization, the counselor is coordinating with a speech-language pathologist, who uses a different set of symbols during communication sessions. The child becomes frustrated when the symbols do not match.

What is the best collaborative action?

A) Ask the pathologist to stop using communication supports outside counseling

B) Compare the systems and agree on consistent, developmentally appropriate symbols that support the child's goals across settings

C) Send all counseling notes so the pathologist can select the child's treatment approach

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Question 40

A client reports nightmares and hypervigilance after an armed robbery and says alcohol is now used nightly to fall asleep. The client minimizes the drinking but has begun missing work and needs increasing amounts for the same effect.

Which assessment approach is most appropriate?

A) Evaluate trauma symptoms and alcohol use together, including their timing, interaction, impairment, and associated risks

B) Assess only trauma because the alcohol use is merely a coping strategy rather than a possible co-occurring problem

C) Defer trauma questions until the client has achieved long-term abstinence

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Question 41

Mina receives individual counseling from a community therapist and brief counseling through her college. She has authorized the two counselors to coordinate because both have begun addressing test anxiety, and the different homework assignments feel contradictory to her.

What should the counselors do?

A) Continue separate plans so each counselor can independently determine which approach works better

B) Include Mina in clarifying each provider's role, align the goals, and agree on how relevant updates will be exchanged

C) Have the college counselor end services automatically because community counseling is more comprehensive

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Question 42

> A counselor begins an intake by reading every question from a checklist without asking why the client sought help. After several minutes, the client says, “None of this gets at what is happening to me.”

What is the best adjustment?

A) Combine necessary structured questions with open-ended inquiry about the client's concerns, goals, strengths, and expectations

B) Continue the checklist without deviation because client-generated information makes intake data unreliable

C) Stop gathering structured information and immediately begin an intervention selected by the client

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Question 43

On an assessment with a mean of 100 and a standard deviation of 15, what does a score of 85 indicate?

A) The score is one standard deviation above the mean

B) The score is one standard deviation below the mean

C) The score is at the 85th percentile

Question 44

> A counselor is about to conduct telehealth from a private home office and notices that a voice-activated smart speaker is on and configured to retain audio snippets in a consumer account. The device is not part of the approved telehealth setup.

What should the counselor do?

A) Continue because consumer smart speakers do not create clinical records

B) Tell the client about the device and treat continued participation as consent to its presence

C) Disable or remove the device and verify a private environment before beginning the session

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Question 45

> During a telehealth session, repeated video freezing prevents the counselor from observing facial expression for much of the meeting. Audio remains clear, and the client participates throughout. The counselor is completing the mental-status portion of the note.

What is the best documentation approach?

A) Record facial expression as normal because the client sounded engaged

B) Document the audiovisual limitation, record what was observable or reported, and avoid conclusions that required a clear image

C) Omit the entire mental-status section because one observational channel was limited

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Question 46

> Soon after starting a new prescription medication, a counselor notices slowed thinking and two episodes of losing track of a client's narrative. The counselor is unsure whether the effects will resolve and has several complex sessions scheduled.

What is the best response?

A) Continue the schedule because medication effects are personal health information rather than a professional concern

B) Tell each client about the medication so clients can decide whether the counselor appears impaired

C) Promptly assess fitness with appropriate medical and supervisory support and arrange coverage or workload changes when needed

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Question 47

Which practice best supports a counselor in maintaining current knowledge of professional ethics?

A) Rely on the ethical principles learned in graduate school unless an employer changes a policy

B) Review informal colleague opinions because ethical standards are primarily established by local custom

C) Review current authoritative standards and guidance, use case-based education, and seek consultation on emerging questions

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Question 48

> A client alternates between avoiding concerns with a roommate and expressing them in an explosive way. The client wants to make a clear request without attacking or withdrawing.

Which in-session intervention is most appropriate?

A) Advise the client to send a written complaint so emotional reactions are avoided.

B) Focus on relaxation only and postpone communication practice until anger no longer occurs.

C) Help the client organize the facts, feeling, and specific request, then rehearse the conversation with feedback.

Question 49

> A former supervisee asks a counselor to certify 100 supervision hours for licensure. The counselor's logs support only 80 hours; the remaining claimed hours were weeks when no supervision meeting occurred.

What should the counselor do?

A) Certify only verifiable hours and address the discrepancy through the required training or board process

B) Sign for all 100 hours because the supervisee performed clinical work during the disputed weeks

C) Backdate additional supervision notes if the supervisee can accurately recall topics they would have discussed

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Question 50

A training program labels an intervention "culturally adapted" but provides little information about the population studied or community involvement. What should a counselor do?

A) Examine the adaptation process, evidence, population limits, and relevant community perspectives before generalizing its use

B) Adopt the intervention because the cultural label indicates that it is appropriate across diverse groups

C) Reject the intervention because cultural adaptations cannot be evaluated with professional evidence

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Question 51

> A principal asks a school counselor for detailed counseling disclosures to help decide whether to suspend a student. No safety or legal exception applies, and school policy permits general educational recommendations but not confidential session content without valid authorization.

How should the school counselor respond?

A) Provide the details because discipline decisions are part of the school's educational mission

B) Ask the student to accept disclosure as a condition of remaining enrolled

C) Protect the counseling content, clarify the counselor's role, and provide only information permitted by policy

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Question 52

> A client with newly emerging persecutory beliefs has stopped eating and drinking because of a conviction that all food and water are poisoned. The client appears weak, has consumed almost nothing for two days, and will not consider outpatient support.

Which level-of-care decision is most appropriate?

A) Arrange immediate medical and psychiatric evaluation because impaired intake and inability to engage safely indicate urgent risk

B) Schedule a routine appointment next week to explore the symbolic meaning of the poisoning belief

C) Mail a personality inventory and wait for the results before considering medical needs

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Question 53

Which counselor action most directly supports client empowerment during a crisis response?

A) Selecting the safest reasonable option for the client to reduce decision burden.

B) Providing understandable choices and supporting the client’s participation in decisions whenever capacity and safety permit.

C) Withholding uncertain information until professionals have reached complete agreement.

Question 54

Which feature best demonstrates internal consistency in a treatment plan?

A) Each presenting problem is assigned the same number of objectives and interventions

B) Each intervention is linked to an objective and supported by the case conceptualization

C) Every objective uses the same review date regardless of urgency or complexity

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Question 55

> A board requires 30 continuing education hours for renewal and permits no more than 10 self-study hours. A counselor has completed 20 self-study hours and 10 qualifying live hours but has not yet submitted the renewal attestation.

What should the counselor do?

A) Count all 30 hours because the overall educational total is sufficient

B) Submit the attestation and promise to replace excess self-study hours after renewal

C) Complete 10 additional qualifying non-self-study hours before attesting to compliance

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Question 56

> Fourteen months after a spouse's death, an adult experiences intense daily yearning, difficulty accepting the death, a sense that part of the self is gone, withdrawal from meaningful activities, and marked impairment. The response substantially exceeds expected practices in the client's cultural and religious context.

Which diagnosis is best supported by the information provided?

A) Acute stress disorder

B) Separation anxiety disorder

C) Prolonged grief disorder

Question 57

> A community organization offers to pay a client's counseling fees. Its director wants progress updates in return. The client agrees to payment but does not authorize any clinical disclosure to the organization.

What should the counselor do?

A) Provide brief updates because anyone paying for treatment becomes part of the care team

B) Refuse the payment because third parties can never fund counseling

C) Clarify the financial arrangement and protect clinical information unless the client validly authorizes disclosure

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Question 58

> In family counseling, parents praise their 15-year-old as “the responsible one” and rely on the teen to supervise younger siblings every evening. The teen reports exhaustion and falling grades but feels guilty asking for change.

What should the counselor facilitate?

A) Make the caregiving pattern visible, validate the teen’s conflict, and help the family redistribute responsibilities in a developmentally appropriate way.

B) Teach the teen more efficient scheduling so the family can preserve the current arrangement.

C) Ask the teen to refuse all future caregiving without discussing the family’s needs.

Question 59

> A client who communicates primarily in Vietnamese arrives for a planned session, but the qualified interpreter cancels unexpectedly. No qualified replacement is available that day, the client declines to proceed without one, and there is no acute safety concern.

Which response best supports continuity?

A) Ask the client's adult child to interpret so that the scheduled treatment plan remains on time

B) Reschedule promptly with a qualified interpreter and confirm how the client can access support before then

C) Conduct the session in English and document that the interpreter's absence was beyond the counselor's control

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Question 60

> A client wants to communicate with a counselor by ordinary text message and assumes messages will be read immediately at all hours. The practice uses a secure portal for routine messages, checks it only during stated business hours, and directs emergencies to designated crisis resources.

What should the counselor clarify during intake?

A) Approved communication methods, privacy risks, expected response times, and procedures for urgent needs

B) The counselor will monitor every messaging channel continuously once counseling begins

C) Digital communication is never appropriate for any administrative or clinical purpose

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Question 61

A client has a personalized crisis plan stored in a phone but says, “When I’m overwhelmed, I can’t find it or remember whom to call.” The client is stable in session and wants to make the plan easier to use.

What should the counselor do?

A) Help the client place the plan and key contacts where they can be reached quickly, then rehearse accessing and following the first step.

B) Ask the client to memorize every contact and coping step before relying on the plan.

C) Keep the only copy of the plan in the clinical record so it cannot be lost.

Question 62

At intake, a couple agreed that the counselor would not promise to keep relationship-relevant information secret from the other partner and would address such information in conjoint work. One partner later emails the counselor about an affair and demands permanent secrecy.

What is the counselor's most appropriate response?

A) Revisit the agreement, help the partner plan prompt disclosure, and avoid continuing conjoint work while colluding with the secret

B) Keep the affair confidential because any message sent individually creates a separate individual-counseling relationship

C) Forward the email immediately to the other partner without first addressing the disclosure process with the sender

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Question 63

A client attends counseling at a partner's request because of frequent cannabis use. The client does not currently want abstinence but is willing to track use and discuss how it affects sleep, spending, and the relationship.

Which initial objective best matches the client's current readiness?

A) Record use and related effects for three weeks, then review the pattern and readiness for change

B) Stop cannabis immediately and ask the partner to report any use before the next session

C) Avoid all discussion of cannabis until the client independently requests a reduction goal

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Question 64

During a telehealth session, the audio connection to the qualified Arabic interpreter repeatedly fails. The client says that continuing in English would be difficult and prefers to stop until effective interpretation is available. The clinic can access a backup interpreter service.

What should the counselor do?

A) Continue in simplified English because the session has already begun

B) Use the backup qualified interpreter service or reschedule according to the client's preference

C) Ask the client to type sensitive answers into a public translation website

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Question 65

A private practitioner notices reluctance to discuss termination with several clients who have met their goals. The counselor is worried about a recent drop in referrals and recognizes that continued fees would stabilize the practice's income.

What is the most appropriate response?

A) Continue sessions until referrals increase because financial stability helps all clients receive consistent care

B) Ask the clients whether they want to continue without discussing the counselor's financial concern

C) Examine the conflict in consultation, evaluate ongoing clinical need, and make transition decisions based on client welfare

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Question 66

A client returns to outpatient counseling after a brief psychiatric hospitalization. The discharge plan includes a new safety plan, changed medication follow-up, and an appointment with a prescriber in three days. The client authorizes coordination, and the outpatient treatment plan has not yet been updated.

What should the counselor do to reestablish continuity?

A) Resume the previous outpatient plan until the prescriber determines whether the hospitalization changes counseling needs

B) Review the discharge information with the client, coordinate authorized follow-up, and reconcile the outpatient plan

C) Replace the outpatient goals with every hospital recommendation without discussing the changes with the client

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Question 67

> Minutes before a complex intake, a counselor learns that a close family member has been hospitalized. The counselor is shaking, cannot retain information from the intake form, and is uncertain about being able to focus.

What should the counselor do?

A) Assess current ability promptly and arrange appropriate coverage or rescheduling rather than beginning while significantly distracted

B) Conduct the intake as planned because personal events should never affect professional availability

C) Begin by telling the new client the full situation so the client can decide whether to continue

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Question 68

In a set of assessment scores, what does the standard deviation describe?

A) The typical spread of scores around the mean

B) The accuracy of a diagnostic cutoff

C) The percentage of items measuring the intended construct

Question 69

> The counselor becomes tearful while a client describes a family loss. The client stops speaking and begins comforting the counselor. The counselor is able to regain composure.

What is the counselor's best response?

A) Accept the comfort because mutual emotional support can strengthen authenticity.

B) Briefly acknowledge the reaction, clarify that caring for the counselor is not the client's responsibility, and explore the impact on the client.

C) Ignore what happened and return immediately to questions about the family loss.

Question 70

> A client speaks rapidly about work, a parent's illness, financial strain, and conflict with a friend, shifting topics before completing any one of them. The client pauses and says, "I don't even know where to start."

Which counselor response is most helpful?

A) Ask a sequence of closed questions so every problem receives equal assessment time.

B) Choose the issue that appears clinically most serious and begin exploring it without interrupting.

C) Briefly summarize the concerns and ask which one would be most useful to focus on today.

Question 71

A professional holds only a state school-counselor credential. State law states that this credential authorizes services within school employment but does not authorize independent clinical mental health practice. The professional plans to open an evening psychotherapy office.

What is the appropriate course of action?

A) Open the office if clients are told that the professional works as a school counselor during the day

B) Refrain from independent clinical practice unless the required clinical license is obtained

C) Use the title wellness consultant while providing the same psychotherapy services

Question 72

A prospective client with severe agoraphobia needs home-based counseling and does not have stable internet access. The counselor provides only office and video sessions, while a local mobile behavioral health team has current openings.

Which response best addresses counselor-client fit?

A) Accept the client for office counseling and make attendance the first treatment goal

B) Offer video counseling because the client can locate public internet when motivation improves

C) Explain the service limitation, coordinate connection with the mobile team, and provide an appropriate bridge if feasible

Question 73

A client living in temporary shelters says a previous office refused intake without a permanent mailing address. The current agency permits alternate contact arrangements and does not require a fixed address for counseling eligibility.

What should the intake counselor do?

A) Explain the agency's actual eligibility policy, identify a reliable contact method with the client, and continue assessing service needs

B) Require the address of the client's most recent shelter even if the client says mail there is not secure

C) Postpone all services until the client obtains permanent housing

Question 74

An applicable ethics code prohibits sexual or romantic relationships with former clients for five years after the last professional contact and requires careful assessment and documentation of potential exploitation after that period. Which conduct is appropriate?

A) Begin a relationship during the five-year period if the former client initiates it

B) Observe the prohibition and, after it ends, retain the burden of showing that no exploitation or harm is likely

C) Treat the five-year mark as automatic approval for a relationship without further evaluation

Question 75

> In family counseling, adult siblings begin discussing resentment about unequal caregiving for their mother. One sibling makes a joke, everyone laughs briefly, and the conversation stops. This pattern has occurred twice in the session.

What is the counselor's best response?

A) Join the humor to reduce tension before introducing a different family topic.

B) Note that humor has appeared when caregiving resentment surfaces and invite each person to describe what happens next internally.

C) Ask the joking sibling to remain silent while the other family members discuss the conflict.

Question 76

> An individual counseling client asks a spouse to attend all future sessions and wants the work to become ongoing couples counseling. The existing consent identifies only the individual as the client and does not address joint records, confidentiality, or the counselor's new role.

What should the counselor do before changing the service?

A) Clarify the clients and roles, assess competence and conflicts, and obtain informed agreement about records and confidentiality

B) Add the spouse without revising consent because the original client requested the change

C) Treat the spouse as a visitor so the counselor can avoid new documentation duties

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Question 77

Which documentation of a clinical consultation is most useful for continuity of care?

A) "Consulted with supervisor" without identifying the issue or resulting plan

B) A verbatim transcript of the entire consultation, including unrelated cases discussed afterward

C) The consultation question, relevant case facts, recommendations received, and the counselor's resulting decision and follow-up

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Question 78

> A counselor receives a realistic video message that appears to show the clinic director ordering an immediate export of the full client roster to a new email address. Clinic policy requires independent confirmation through a known channel before any unusual bulk export.

What should the counselor do?

A) Export the roster because a video image is stronger authentication than an email signature

B) Send a sample of ten client records first to determine whether the address is legitimate

C) Withhold the export and verify the request with the director through the approved independent channel

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Question 79

> Elena wants to enroll in a parenting-support program that requires an online intake. She has no email address, limited digital experience, and access only to a basic mobile telephone. The program also offers staff-assisted intake by telephone, but this option is not shown on its website.

What is the counselor's best referral support?

A) Tell Elena to create an email account and complete the online process independently

B) Verify the staff-assisted option and help Elena connect with it while allowing her to provide her own information

C) Complete the online form in Elena's name using an agency email account

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Question 80

> Caleb tells his counselor that his pharmacy is out of a prescribed medication and that he has two doses remaining. He reports no current symptoms and asks whether he should skip doses to make the supply last. The counselor has authorization to contact the prescriber and pharmacy.

What is the counselor's most appropriate response?

A) Tell Caleb to take half-doses so the remaining supply lasts longer

B) Document the access problem and help Caleb promptly contact the prescriber and pharmacy for verified continuity instructions

C) Recommend borrowing the same medication from a family member until the pharmacy restocks it

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Question 81

> A client requests a woman counselor because discussing sexual concerns with a man would conflict with the client's religious privacy practices. The agency has women counselors available, although the first available appointment may be one week later.

What is the best intake response?

A) Challenge the request as avoidance before discussing scheduling or service options

B) Assume that every member of the client's religious community has the same preference

C) Explore the client's stated preference, explain available options and wait times, and arrange a suitable match when feasible

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Question 82

A client reports taking more prescribed alprazolam than directed, drinking alcohol most evenings, and having two recent episodes of memory loss. The client becomes shaky when attempting to reduce the medication and has not told the prescriber about the alcohol use.

What is the best assessment response?

A) Assess amounts, timing, withdrawal history, co-use risks, and current medical status, and facilitate prompt medical or substance-use evaluation

B) Recommend that the client abruptly stop both substances at home before any further assessment

C) Focus only on the original anxiety symptoms because prescribed medication use cannot constitute a substance-related concern

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Question 83

A client wants to respond more assertively to a supervisor but reports that coworkers who challenged this supervisor were assigned worse shifts. The client cannot risk losing work hours and is uncomfortable with direct confrontation.

How should the counselor adapt assertiveness work?

A) Explore a range of lower-risk options, such as documenting concerns, consulting a trusted ally, or making a limited request, and let the client choose.

B) Rehearse a direct confrontation because avoiding it will reinforce the client’s fear.

C) Advise the client to accept the situation because assertiveness is unsafe in this workplace.

Question 84

A counselor loses a clinic laptop while traveling. The laptop is encrypted and supports remote wipe, but an active portal session may still be open. Security policy requires immediate reporting, session revocation, remote wipe, and documented risk assessment.

What should the counselor do?

A) Report the loss immediately and activate the required containment and assessment steps

B) Wait 24 hours for the laptop to be returned because encryption eliminates all risk

C) Change only the counselor's email password because the clinical files are encrypted

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Question 85

> A client with limited reading proficiency signs a lengthy intake form quickly and then cannot explain the basic purpose of counseling or the stated limits of confidentiality. The client says, “I sign whatever offices give me.”

What should the counselor do?

A) Treat the signature as sufficient proof of informed consent and begin the session

B) Ask a family member to summarize the form without first asking the client's preference

C) Review the material orally in plain language, offer accessible alternatives, check understanding, and invite questions before proceeding

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Question 86

> A client says, “When you stay quiet after I answer, it feels cold, like you’re testing me.” The counselor had intended the silence to provide room for reflection.

What should the counselor do?

A) Acknowledge the impact, clarify the intention briefly, and ask what kind of pacing would feel more supportive.

B) Explain that therapeutic silence is an established counseling skill and encourage the client to tolerate it.

C) Stop using any silence with this client so the issue does not recur.

Question 87

> Imani is moving from a domestic-violence shelter to permanent housing and has authorized coordination among the counselor, a housing advocate, and a nonprofit that has temporarily fostered her cat. Imani says reuniting with the cat is important to feeling settled, but the housing plan does not yet address the building's pet requirements.

What should the counselor facilitate?

A) Add Imani's pet-reunification goal to the coordination plan and clarify the advocate's and nonprofit's related next steps

B) Exclude the cat from planning because animal care is unrelated to emotional well-being

C) Ask the nonprofit to return the cat immediately before the housing requirements are confirmed

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Question 88

> Rania requests counseling in Levantine Arabic. At intake, she says that the assigned interpreter's dialect is difficult for her to understand and that she is missing parts of the conversation. The interpreter is qualified but does not speak her preferred dialect.

What should the counselor do?

A) Continue because interpreter credentials matter more than dialect differences

B) Ask Rania to guess at unfamiliar words and correct misunderstandings after the intake

C) Pause the intake and arrange a qualified interpreter who can communicate effectively in Rania's preferred dialect

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Question 89

> A 68-year-old client reports gradually worsening forgetfulness over 18 months, including missed bill payments and repeated questions. The client's spouse also describes reduced interest and low mood. The client takes several medications and has not had a recent medical or cognitive evaluation.

Which assessment plan is most appropriate?

A) Attribute all memory concerns to depression because low mood can affect concentration

B) Diagnose a major neurocognitive disorder solely from the spouse's report of missed bills

C) Assess cognition, mood, daily functioning, medication and medical factors, and obtain relevant collateral information before forming a diagnosis

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Question 90

> A client treated for depression has lower symptom scores after ten weeks but has begun missing more work and reports that morning routines remain unmanageable. The client values employment stability more than further symptom reduction.

What is the best response to the mixed outcome pattern?

A) Review what produced symptom change and revise the plan to address the unresolved functional goal

B) Continue the plan unchanged because symptom reduction is the most objective indicator of success

C) End treatment because improvement in one outcome means the intervention has been effective overall

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Question 91

A middle school student has begun refusing school. Interviews show that anxiety rises mainly on the bus, where peers have repeatedly harassed the student. The student wants to return, and the school can provide alternate transportation while addressing the harassment.

Which initial plan best integrates the available information?

A) Require use of the same bus so avoidance does not reinforce the student's anxiety

B) Focus exclusively on relaxation because school transportation is outside counseling goals

C) Pair a supported return plan with alternate transportation and coping objectives while the school addresses harassment

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Question 92

A family has maintained stable housing for six months with help from a wraparound team. The family wants fewer formal services but still needs occasional help navigating benefits. A relative and a neighborhood organization can provide agreed support.

Which step-down plan best supports sustainable continuity?

A) Reduce services in stages, transfer specific tasks to chosen supports, and define indicators for renewed formal help

B) End all team services at once because stable housing shows that wraparound care is no longer needed

C) Keep every provider involved at the same intensity to prevent the possibility of future housing problems

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Question 93

At intake, an agency invites clients to allow de-identified questionnaire data to be used in a research project. Participation is optional, declining has no effect on counseling eligibility, and the research requires a separate consent process.

What should the counselor communicate?

A) Research participation is required because the questionnaires are completed during intake

B) Refusal may reduce access to counseling even though agency policy says eligibility is unaffected

C) Counseling and research are separate choices, and the client may decline research without losing services

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Question 94

A client obtains an extremely elevated score on an anxiety inventory but appears confused when discussing several items. The client later explains that negatively worded items were interpreted in the opposite way because of reading difficulty.

How should the counselor use the result?

A) Diagnose an anxiety disorder because numerical scores are more objective than client explanations

B) Clarify item comprehension, consider whether the administration was valid, and integrate other interview and observational data before interpreting the score

C) Discard all formal assessment methods permanently because this administration may be invalid

Question 95

An integrated care client with irritable bowel syndrome and anxiety avoids eating before leaving home, worries about urgent symptoms, and has become socially isolated. The medical team has ruled out an acute condition, and the client wants to resume shared meals and community activities.

Which plan best reflects a biopsychosocial conceptualization?

A) Focus only on anxious thoughts because medical evaluation found no acute disease

B) Defer counseling until gastrointestinal symptoms disappear through medical treatment

C) Coordinate relevant medical care while targeting avoidance, coping, nutrition routines, and gradual return to valued activities

Question 96

If an applicable access rule requires a readily producible electronic copy when a client requests one, which response is most appropriate?

A) Provide the copy through a secure available method and charge only any permitted reasonable cost

B) Require paper copies at a higher fee because paper is easier for the practice to track

C) Refuse electronic access unless the client purchases the same record software used by the practice

Question 97

For eight months, a client has been preoccupied with having a serious illness despite minimal physical symptoms and repeated reassuring medical evaluations. The client spends hours checking the body and researching diseases, and the anxiety causes substantial impairment.

Which diagnosis is most consistent with this information?

A) Somatic symptom disorder

B) Factitious disorder imposed on self

C) Illness anxiety disorder

Question 98

A graduate student seeks counseling for perfectionism that delays completion of a thesis. Both a cognitive-behavioral plan and an acceptance-based plan could address the problem. The student prefers an acceptance-based approach and wants progress measured by submitting drafts on schedule.

Which planning decision is most appropriate?

A) Select the cognitive-behavioral plan because counselor preference should resolve choices between suitable approaches

B) Combine every technique from both approaches so that no potentially helpful intervention is omitted

C) Develop the acceptable evidence-informed plan around scheduled draft submission and monitor whether it produces progress

Question 99

A client revokes a written authorization that allowed monthly summaries to be sent to a disability case manager. One summary was lawfully sent before the revocation. Clinic policy makes revocation effective for future disclosures when received and documented.

How should the counselor respond?

A) Ask the case manager to destroy the earlier summary because revocation automatically cancels past disclosures

B) Document the revocation, stop future summaries, and explain that the prior authorized disclosure cannot be undone

C) Continue sending summaries until the authorization's original expiration date because it was valid when signed

Question 100

Keisha authorizes a neuropsychologist to send her counselor a recent evaluation before their next session. The report is 30 pages long and includes broad testing results. Keisha's counseling goal is to improve organization at work, and she has not asked the counselor to interpret the full test battery.

How should the counselor use the report in preparing for the session?

A) Replace the existing counseling plan with every recommendation in the evaluation

B) Set the report aside because a counselor did not administer the tests

C) Review findings and recommendations relevant to Keisha's goal and note questions that require clarification

Question 101

What does attainable mean when evaluating a treatment objective?

A) The objective is realistically achievable from the client's baseline with available time and supports

B) The objective can be completed without discomfort, setbacks, or assistance from other people

C) The objective reflects the counselor's ideal outcome even if the client doubts it can be reached

Question 102

An aptitude test is administered to applicants, and scores are later compared with their job performance after one year. This procedure primarily evaluates which property?

A) Face validity

B) Predictive criterion validity

C) Internal consistency

Question 103

> A practice receives formal notice of a malpractice claim involving a former client. The practice's legal-hold procedure requires preservation of potentially relevant records, but those records are scheduled for routine destruction next week.

What should the records custodian do?

A) Follow the normal schedule because changing it after a claim may appear suspicious

B) Suspend destruction of the relevant records, preserve them securely, and document the legal hold

C) Give the records to the former client so the practice no longer has a retention obligation

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Question 104

> Several group members blame one member for “making every session tense.” The targeted member becomes quiet, while the others unite around criticism and stop examining their own contributions.

What should the group leader do?

A) Interrupt the scapegoating, describe the group-wide pattern, and invite each member to examine a personal contribution to the tension.

B) Ask the targeted member to explain why multiple people have reached the same conclusion.

C) Let the criticism continue because shared agreement indicates that the group’s feedback is accurate.

Question 105

> A client believes the person conducting the intake is a fully licensed counselor. The clinician is actually a graduate intern working under the direct supervision of a licensed counselor, and clients may request assignment to another available clinician.

What should be explained before the client agrees to services?

A) Only the intern's academic program, because supervision and alternatives are administrative details

B) Nothing about training status as long as the supervisor reviews the file

C) The intern's status and role, the supervisor's involvement, relevant confidentiality implications, and available service options

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Question 106

A 14-year-old is frequently late to first period after staying online past the agreed bedtime. The adolescent helps create a plan, values weekend recreation with family, and can place the device in a common charging area. The caregiver can provide one consistent morning cue.

Which behavioral plan is most appropriate?

A) Define the charging and morning routines, reinforce on-time days with agreed weekend recreation, and track arrival time

B) Remove all device access for an unspecified period after each late arrival and allow the caregiver to change the consequence

C) Ask the adolescent to promise better choices and discuss lateness only when a school complaint occurs

---

Question 107

A solo practitioner is developing a plan for sudden incapacity or death. The practice holds active client records, future appointments, and advance payments, but no one else currently has authority to manage them.

Which plan is most appropriate?

A) Designate a qualified records custodian and create secure procedures for notice, continuity, access, and financial obligations

B) Give every client the practitioner's personal passwords so clients can retrieve their own files if needed

C) Leave instructions to destroy all records immediately so no successor assumes responsibility

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Question 108

A counselor has facilitated several bereavement groups in succession. The counselor now avoids calls from friends who are grieving, feels guilty when enjoying family events, and has difficulty remaining emotionally present during group members' stories.

What is the best response to these changes?

A) Evaluate the impact with supervision, use appropriate personal support, and consider rotating duties or reducing cumulative grief exposure

B) Stop all contact with grieving people outside work so emotional energy remains available for clients

C) Leave bereavement counseling permanently because reduced emotional presence cannot be restored

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Question 109

A counselor has a severe dog allergy that cannot be adequately managed in the available office. A prospective client requires a trained service dog during sessions. Another qualified counselor in an accessible suite can accept the client immediately.

Which fit decision is most appropriate?

A) Ask the client to attend without the service dog because counseling sessions are brief

B) Explain the environmental limitation and facilitate care with the available qualified counselor

C) Accept the client and attempt sessions until an allergic reaction actually interrupts treatment

---

Question 110

Which approach best supports compliant text-based communication with counseling clients?

A) Use ordinary personal texting whenever a client states that convenience is the highest priority

B) Use an approved secure channel, establish response and emergency expectations, and verify intended recipients

C) Copy every text exchange to all treatment-team members so no message is overlooked

---

Question 111

A community clinic finds that hourly workers discontinue counseling more often than salaried clients. Interviews indicate that unpaid time away from work and limited appointment hours are major barriers.

Which professional growth and advocacy response is best?

A) Teach hourly workers time-management strategies before changing clinic operations

B) Maintain the schedule because offering identical hours to everyone is the fairest policy

C) Learn about economic access barriers and use client data to advocate for feasible scheduling or service alternatives

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Question 112

A competent adult client's sibling presents a power of attorney limited to banking and property matters. The sibling demands authority to consent to counseling and receive clinical updates, but the document grants no health-care decision-making power.

What should the counselor do?

A) Treat the sibling as the client's clinical representative because any power of attorney covers health decisions

B) Verify the document's limits and obtain treatment and disclosure decisions from the competent client

C) Release updates to the sibling but ask the client to consent separately to treatment

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Question 113

> A counselor is in network with a health plan whose contract prohibits collecting more than the plan-assigned copayment, deductible, and coinsurance. The counselor wants to charge clients an additional $30 per covered session to offset the contracted rate.

What should the counselor do?

A) Follow the network agreement, correct any excess charges, and explain client responsibility accurately

B) Collect the extra amount if clients sign a separate private-pay agreement

C) Label the charge an administrative fee so it falls outside the network contract

---

Question 114

> A client responds to mistakes with statements such as, “I’m useless and deserve to feel bad.” The client fears that a kinder internal response would eliminate accountability and reduce effort.

Which intervention best addresses this concern?

A) Tell the client that self-criticism is irrational and should be replaced with praise.

B) Avoid self-compassion because the client believes criticism supports performance.

C) Practice a balanced compassionate response that acknowledges the mistake and a corrective action, then compare its effect on motivation with the harsh response.

Question 115

> An agency proposes using a brief symptom screener score as the sole rule for discharging clients. A counselor is trained to administer the screener but knows that it was designed to support, not replace, broader clinical evaluation.

How should the counselor respond?

A) Follow the rule because standardized scores are less biased than counselor judgment

B) Alter borderline scores when continued counseling appears beneficial

C) Explain the instrument's limits, advocate for appropriate interpretation, and seek assessment consultation if needed

---

Question 116

> One partner insists that sharing all phone passwords proves commitment. The other partner experiences the demand as controlling but worries that refusing will be seen as evidence of dishonesty. Both ask the counselor to define what trusting couples should do.

What is the counselor's best intervention?

A) Recommend password sharing because mutual transparency reduces suspicion.

B) Explore each partner's meanings of trust, privacy, and safety, then facilitate a mutually consented boundary rather than prescribe one.

C) Recommend separate phones and accounts so the issue cannot affect the relationship.

Question 117

> A Muslim client explains that fasting during Ramadan, late evening family meals, and early prayer will temporarily change sleep and activity patterns. The client wants to continue counseling for depression and asks that the plan reflect these personally meaningful practices.

How should the counselor adapt the plan?

A) Collaborate on realistic sleep, activity, and session objectives that fit the client's stated observance and preferences

B) Treat fasting as a depressive symptom because it changes the client's usual eating and sleeping schedule

C) Keep all objectives unchanged because cultural and religious practices should not affect clinical planning

---

Question 118

> A counselor notices growing physical attraction to a client and has started dressing differently on session days and reviewing the client's messages more often than clinically necessary. No boundary violation has yet occurred.

What should the counselor do?

A) Seek supervision promptly, reinforce professional boundaries, and arrange a careful transfer if objectivity cannot be maintained

B) Tell the client about the attraction so both parties can monitor its effect on counseling

C) Ignore the reaction unless the client expresses attraction or a boundary violation occurs

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Question 119

> At an emergency shelter after a severe storm, a client is handed several forms and pages of instructions. The client stares at the papers and says, “I can’t take in any of this.” Immediate safety and basic medical needs have already been addressed.

What should the counselor do?

A) Ask which immediate concern matters most, address one step at a time, and check the client’s understanding before adding information.

B) Read every form aloud so the client receives all available information during the meeting.

C) Set the paperwork aside until the client appears fully calm and requests it independently.

Question 120

> A client shares a mobile phone with several relatives and asks the clinic not to leave voicemail or display the clinic name in text notifications. The clinic can use neutral appointment reminders or contact the client through a secure portal.

What is the most appropriate intake action?

A) Use standard voicemail because all clients must receive identical reminder messages

B) Document the client's communication preferences, explain relevant privacy limitations, and use an available safer contact method

C) Stop all appointment reminders without discussing how the client prefers to receive scheduling information

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Question 121

> A Native American client says that meeting with a traditional healer is important and asks whether those practices can be used alongside counseling. The client wants to decide what, if anything, is shared between helpers.

How should the counselor respond?

A) Explain that the client must choose one approach so the interventions do not conflict.

B) Explore how the client wants the approaches to complement each other and coordinate only with the client’s informed permission.

C) Contact the healer for cultural guidance before continuing counseling with the client.

Question 122

During conflict, one partner becomes flooded and goes silent to calm down. The other follows from room to room, interpreting the silence as abandonment. Both want a way to interrupt this cycle without avoiding the issue.

Which intervention should the counselor facilitate?

A) Instruct the silent partner to remain in the conversation until the other feels reassured.

B) Tell the pursuing partner to stop raising concerns that could overwhelm the other.

C) Help the couple rehearse a time-limited pause that includes a clear statement of when they will return to the conversation.

Question 123

A counselor begins outreach to seasonal farmworkers and initially interprets missed appointments as low commitment. A community liaison explains that unpredictable work demands, transportation, and fear of lost wages shape attendance. The counselor has little knowledge of agricultural labor conditions.

Which professional development action is most appropriate?

A) Teach clients that regular attendance is necessary before adapting any services

B) Learn from qualified community and professional sources, examine the counselor's assumptions, and help the agency design feasible access options

C) Refer farmworkers to counselors from the same occupational background because training cannot address the experience gap

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Question 124

Jamal receives a long-acting medication at a specialty clinic. He missed the latest appointment because the clinic moved, and the old telephone number now plays a disconnected message. Jamal reports no acute symptoms, and his authorization permits coordination with the clinic and prescriber.

What should the counselor do?

A) Verify the clinic's current access information and help Jamal reconnect promptly with the medication team

B) Tell Jamal that missing one appointment is unlikely to matter and wait for the clinic to contact him

C) Recommend an oral substitute until Jamal locates the new clinic

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Question 125

A counselor leads a court-mandated group and feels challenged by one member's frequent objections. The counselor has begun responding with sarcasm and reminding the member that attendance is not voluntary.

What should the counselor do?

A) Recognize the misuse of positional power, stop the sarcastic responses, and use supervision to restore respectful and consistent facilitation

B) Continue emphasizing the mandate because authority is the most effective response to group resistance

C) Remove the member from the group because objections show unwillingness to benefit from counseling

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Question 126

Which statement best describes confidentiality when a client reports a completed illegal act that presents no ongoing threat and no consent, legal compulsion, or other exception applies?

A) Confirm only that the person is a client because that fact is less sensitive than session content

B) Maintain confidentiality and require a valid basis before disclosing any protected information

C) Describe the act without the client's name because police already know whom they are investigating

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Question 127

An evening parenting program has high dropout among single parents. Participants report that sessions overlap with shift work and that no child care is available. Staff members have been describing the departures as lack of motivation.

Which response best combines staff development and advocacy?

A) Train staff to explain the attendance expectations more firmly before enrollment

B) Learn about the practical barriers with participants and advocate for scheduling, child-care, or delivery options that address them

C) Limit future enrollment to parents who can guarantee attendance at the current time

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Question 128

A 13-year-old has significant weight loss and nutritional deficiency because many foods are avoided for their texture and smell. The adolescent expresses no fear of weight gain, no body-image disturbance, and no desire to become thinner. A medical condition does not fully explain the restriction.

Which diagnosis is most consistent with this presentation?

A) Anorexia nervosa

B) Avoidant/restrictive food intake disorder

C) Body dysmorphic disorder

Question 129

A client seeking help for insomnia works rotating day and night shifts that cannot be changed. The initial objective requires the same bedtime and wake time every day, and the client has been unable to follow it despite using other sleep strategies consistently.

What is the most appropriate plan revision?

A) Develop shift-specific sleep routines and a feasible anchor period while continuing to monitor sleep outcomes

B) Retain the fixed schedule because modifying an evidence-informed plan would reward nonadherence

C) End insomnia work until the client obtains employment with a stable schedule

---

Question 130

A telehealth consent form is returned with the client's typed name, but metadata show that it came from the partner's email account. The partner admits completing the form for the competent adult client. Practice policy requires identity verification and the client's own valid consent.

What should the counselor do?

A) Verify the client's identity and obtain the client's valid consent before beginning telehealth services

B) Accept the form because a partner may sign any document that benefits the client

C) Begin treatment and ask the client to ratify the form only if a dispute later occurs

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Question 131

A client completing counseling for anger outbursts has an upcoming co-parenting meeting, a situation that previously led to escalation. The client can recognize jaw tension as an early cue and has successfully used planned pauses during recent disagreements.

Which maintenance plan best prepares the client for the meeting?

A) Avoid all direct co-parenting meetings indefinitely so that anger cannot recur

B) Depend on motivation to remain calm and contact the counselor only if another outburst occurs

C) Link the early cue to a rehearsed pause, identify support after the meeting, and define when to seek follow-up

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Question 132

Before a session, a counselor reviews a symptom measure the client completed at home. One required item is blank, and the scoring instructions do not permit the counselor to treat a missing response as zero. The incomplete measure has not yet been discussed with the client.

How should the counselor prepare to use the measure?

A) Score the blank item as zero because that produces the most conservative total

B) Discard the measure and avoid mentioning it because it cannot be scored immediately

C) Flag the missing item, plan to clarify it with the client, and interpret the measure only as allowed after clarification

---

Question 133

A counselor begins a prescribed medication that causes marked slowed thinking and has already led to two serious documentation errors. The counselor's physician expects the effect to continue while the dosage is adjusted.

What is the counselor's most professionally accountable response?

A) Continue the full caseload because a valid prescription rules out professional impairment

B) Limit or suspend affected duties, arrange client coverage, and resume only when competent performance is restored

C) Ask clients to overlook mistakes temporarily because stopping services could inconvenience them

---

Question 134

A counselor grants larger fee reductions to clients the counselor personally likes and smaller reductions to similarly situated clients. The practice has no written sliding-fee criteria.

What should the counselor do?

A) Continue using personal judgment because flexibility is the purpose of a sliding fee

B) Establish transparent, relevant criteria and apply and document them consistently

C) Eliminate reduced fees for all current clients without notice to avoid unequal treatment

---

Question 135

Because writing is physically painful, Rowan and the counselor agreed that Rowan could complete a between-session reflection as a voice memo rather than in a journal. Rowan brings the recording to the next session and discusses two themes that emerged.

Which progress-note entry is most appropriate?

A) Record that Rowan completed the reflection in the agreed audio format, summarize the relevant themes and response, and state the next plan

B) Document that the written homework was not completed and omit the voice memo because it used a different format

C) Describe the full contents of the recording even when portions were unrelated to the counseling goal

---

Question 136

> A counselor's draft note states, "The client used paced breathing and reported lower tension" under the heading Intervention. It does not identify what the counselor did during the exercise. The note has not yet been signed.

How should the counselor revise the entry?

A) Keep the statement as written because the client's action is sufficient to identify the intervention

B) Record the counselor's breathing instruction under the intervention and the client's practice and reported change under the response

C) Move the entire statement to the plan section because breathing should occur between sessions

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Question 137

Which element is most characteristic of an aftercare plan?

A) A detailed reconstruction of every topic discussed during active treatment

B) Specific post-treatment supports, responsibilities, and contingencies for maintaining gains

C) A statement that the client may contact any provider if a future problem develops

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Question 138

Which case-conference agenda is most likely to produce effective stakeholder coordination?

A) A complete retelling of the client's history followed by open discussion with no assigned follow-up

B) Separate presentations ranked by each participant's professional seniority

C) Client-defined goals, decisions needed, participant responsibilities, and a method and date for follow-up

---

Question 139

When is a structured round especially useful in group counseling?

A) When the leader wants every member to agree with a single interpretation.

B) When a focused prompt can help balance participation and briefly hear from each willing member.

C) When one member needs uninterrupted time for an extended individual counseling exchange.

Question 140

> A secure messaging agreement states that the counselor reviews portal messages on business days, responds within two business days, and does not provide emergency coverage through the portal. A client sends a nonurgent scheduling question late Friday evening.

What should the counselor do?

A) Respond within the disclosed service window and reinforce the messaging expectations if needed

B) Treat the message as an emergency because every after-hours client contact requires immediate action

C) Close the portal account because the client sent a message outside business hours

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Question 141

> The parent of a child receiving counseling reports untreated panic symptoms and asks the child's counselor to provide therapy for the parent during the child's sessions. The parent is willing to seek a separate provider, and the child's progress depends partly on the parent's ability to follow the home plan.

Which continuity response is most appropriate?

A) Treat the parent's panic within the child's sessions because caregiver functioning affects the child

B) Tell the parent to find care independently so the child's counseling remains completely separate

C) Provide an appropriate adult referral and, with permission, coordinate only the caregiver support relevant to the child's plan

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Question 142

What is the primary purpose of an exception question in solution-focused counseling?

A) To determine whether the client is minimizing how often the problem occurs.

B) To identify times the problem is absent or less intense and examine resources associated with those times.

C) To uncover the early experience that originally caused the current problem.

Question 143

> A client wants to stop escalating minor disagreements into angry outbursts. The client usually notices the problem only after shouting and says, “It happens too fast to change.”

Which in-session intervention would be most useful?

A) Ask the client to suppress angry thoughts as soon as an argument begins.

B) Focus exclusively on the consequences of past outbursts to increase motivation.

C) Map a recent episode step by step to identify triggers, early body cues, actions, and points for a different response.

Question 144

A client converses comfortably in English but requests a qualified interpreter for complex counseling discussions. What is the best response?

A) Decline because conversational English shows that interpretation is unnecessary

B) Ask a bilingual relative to remain available only if the client becomes confused

C) Arrange qualified interpretation in the client's preferred language rather than assuming conversational fluency is sufficient

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Question 145

An advertisement promises a "certified trauma expert" title after a six-hour self-paced course. It gives no information about instructor qualifications, skill assessment, supervised practice, or recognition by a professional body.

How should a counselor evaluate this opportunity?

A) Compare the course with credible competency standards, evidence, instructor qualifications, and opportunities to demonstrate skill

B) Enroll because use of the word certified means the title is recognized across counseling settings

C) Enroll if the counselor already treats trauma because experience will supply all missing training components

Question 146

A counselor in stable recovery from stimulant use begins treating a client who describes recent use in vivid detail. After several sessions, the counselor notices unexpected cravings and increasing difficulty concentrating during the client's appointments.

What should the counselor do?

A) Promptly use supervision and recovery supports to assess functioning and arrange transfer if competent care cannot be maintained

B) Disclose the counselor's recovery history so the client can avoid descriptions that trigger cravings

C) Continue without consultation because personal recovery should make the counselor more effective with substance-use concerns

Question 147

A client with panic disorder and a seizure disorder is considering interoceptive exposure exercises that deliberately produce dizziness and rapid breathing. The counselor has no information about whether those exercises are medically appropriate for this client.

How should the counselor address this issue in the plan?

A) Begin the standard exercises because temporary physical sensations are the intended part of treatment

B) Coordinate appropriate medical input and use suitable plan components while clarifying which exercises are safe

C) Remove all exposure-based goals permanently because any seizure history rules out behavioral treatment

Question 148

A client who lives in the same small community as the counselor asks for the counselor's street and the school attended by the counselor's children. The client says, "It would help me know you understand people like us."

What is the most appropriate response?

A) Provide the information because shared-community disclosure is necessary for cultural trust.

B) Decline the questions without discussion because any personal inquiry threatens professional boundaries.

C) Explore what the information would mean to the client, explain the privacy limit, and discuss safer ways to address the need for understanding.

Question 149

A client at a community agency begins shouting after learning that an assistance request was denied. The client is not threatening anyone but repeatedly interrupts and demands that the counselor "fix it now."

Which response is most likely to support de-escalation?

A) Explain all reasons the request was denied so the client understands the agency's decision.

B) Tell the client that the meeting will end unless the interruptions stop immediately.

C) Lower the pace, reflect the urgent need behind the anger, and offer a choice between reviewing an appeal step or identifying another resource.

Question 150

Before a client's first appointment, a roommate sends the counselor a detailed email describing the client's behavior and asks for confirmation that the client has scheduled counseling. The client has not authorized contact with the roommate.

What is the most appropriate intake response?

A) Confirm the appointment but decline to discuss the content of future sessions

B) Avoid confirming client status, preserve the information according to applicable practice procedures, and discuss collateral involvement with the client if the intake occurs

C) Reply with the counselor's preliminary diagnosis so the roommate can provide additional observations

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Question 151

> A client in career counseling met an objective of submitting four job applications weekly ahead of schedule. The broader employment goal remains active, and the client now reports anxiety about interviewing.

How should the plan be revised?

A) Continue the application objective unchanged until the original review date to preserve consistency

B) Document the gain and collaboratively add an interview-preparation objective while maintaining useful application habits

C) Close the employment goal because completing one objective demonstrates readiness to manage the remaining steps alone

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Question 152

> A refugee client becomes tense when automated appointment texts arrive and explains that electronic messages from institutions were used for surveillance in the client's country of origin. The clinic offers telephone, email, text, or no automated reminders.

What should the counselor do?

A) Explore the client's concern, explain how reminder information is handled, and let the client choose an acceptable reminder method.

B) Continue texts because repeated neutral messages will gradually correct the client's fear.

C) Disable every electronic service without discussion to protect the client from further distress.

Question 153

> While reviewing an unsigned note, a counselor realizes that a sentence placed inside quotation marks is a paraphrase of the client's statement rather than the client's exact words. The paraphrase is otherwise accurate.

What should the counselor do before signing the note?

A) Remove the quotation marks and identify the content as the client's reported meaning

B) Keep the quotation marks because the paraphrase accurately captures the client's intent

C) Delete the statement because only exact quotations may appear in a progress note

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Question 154

> A 16-year-old client wants to continue both a counseling skills group and the school basketball team, but their schedules overlap twice each month. The client and guardian authorize the counselor to coordinate scheduling with the school counselor and coach. The client does not want clinical details shared with the coach.

What coordination approach best supports the client?

A) Compare schedule alternatives with the client and authorized stakeholders while limiting communication to the participation issue

B) Ask the client to leave the team because counseling should take precedence over extracurricular activities

C) Explain the client's symptoms to the coach so the coach understands why schedule flexibility is necessary

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Question 155

Which procedure most directly evaluates interrater reliability for an observational assessment?

A) Administering the same instrument to the same clients again two weeks later

B) Comparing scores assigned independently by two trained observers watching the same behavior

C) Correlating the assessment with a different measure of the same construct

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Question 156

What is the main therapeutic function of a minimal encourager such as “Mm-hmm” or “Go on”?

A) To communicate attention and invite the client to continue without directing the content.

B) To indicate that the counselor agrees with the client’s interpretation.

C) To move the client toward a topic the counselor considers more important.

Question 157

> A 15-year-old begins counseling with a parent present. The parent says, “I expect to hear everything discussed here,” and the adolescent looks alarmed. The agency’s consent form allows private adolescent sessions and specifies the information that must be disclosed for safety.

What should the counselor do?

A) Promise the adolescent complete confidentiality so honest counseling can begin.

B) Review the confidentiality agreement and its safety limits with both, explain how updates can be handled, and invite questions before proceeding.

C) Ask the parent to leave immediately and discuss confidentiality only with the adolescent.

Question 158

A client has persistent difficulty discarding possessions regardless of value, intense distress when attempting to discard, and severe clutter that blocks the kitchen and one exit. The accumulation is not attributable to another medical condition or to another mental disorder.

Which diagnosis is best supported by these findings?

A) Hoarding disorder

B) Obsessive-compulsive personality disorder

C) Specific phobia, situational type

Question 159

A client with no acute safety concern says rising out-of-pocket costs require ending counseling earlier than planned. The client has made partial progress and wants to preserve gains but cannot continue at the current fee.

What is the most appropriate response?

A) Review progress and remaining needs, create a focused ending plan, and offer affordable continuation or reentry options

B) Insist that the client complete the original course because early termination may weaken treatment gains

C) End immediately without further planning because financial decisions fall outside the counseling relationship

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Question 160

A licensed counselor wants to volunteer clinical services at a disaster shelter. The counselor's employer policy covers only work performed for the employer, and the shelter requires volunteers to confirm independent liability coverage and compliance with local scope rules.

What should the counselor do before providing counseling?

A) Rely on the active license because licensure automatically supplies liability coverage

B) Begin immediately and purchase coverage only if a client later files a complaint

C) Verify appropriate coverage, authorization, and scope compliance for the volunteer setting

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Question 161

A client with a chronic psychotic disorder is considering a voluntary outpatient program. The client can state a choice but gives inconsistent explanations of the program and how its risks and benefits apply personally. The family insists that the diagnosis itself proves incapacity.

How should the counselor assess decision-making capacity?

A) Accept the family's conclusion because a psychotic disorder automatically eliminates capacity

B) Evaluate the client's ability to understand information, appreciate its personal relevance, reason about options, and communicate a stable choice

C) Determine capacity solely from whether the client agrees with the counselor's recommendation

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Question 162

A client repeatedly arrives late. The counselor, who grew up in a household where lateness was treated as disrespect, begins ending this client's sessions earlier than the agency attendance policy requires and feels satisfied when the client becomes upset.

What is the most appropriate response?

A) Keep the stricter consequence because the client's emotional reaction shows that the limit is effective

B) Examine the personal meaning of lateness, consult as needed, repair the inconsistency, and apply policy based on clinical facts

C) Avoid addressing attendance in the future because the counselor can no longer set an objective limit

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Question 163

A university conduct program requires a student to attend six counseling sessions addressing consequences of alcohol use. The student does not agree that alcohol is the main problem but wants help improving grades after several missed classes. The program permits individualized goals in addition to the required focus.

Which initial plan best balances the mandate and collaboration?

A) Explain the required focus and confidentiality limits, then link alcohol-related review with the student's academic goal

B) Use all six sessions only for alcohol education because required services cannot include client-selected goals

C) Avoid discussing alcohol until the student independently identifies it as a problem

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Question 164

> Andre receives hemodialysis three mornings per week and is often fatigued afterward. He has authorized coordination between his counselor and the dialysis social worker. Several afternoon counseling appointments have been canceled on treatment days, but Andre wants to continue counseling.

Which coordination response is most appropriate?

A) With Andre's input, align counseling times with the dialysis schedule and establish a continuity plan for treatment-related disruptions

B) Close Andre's counseling case because medical treatment makes regular attendance uncertain

C) Ask the dialysis social worker to decide when Andre is emotionally ready for counseling

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Question 165

> A counselor observes a colleague smelling strongly of alcohol, speaking unclearly, and preparing to conduct a group session. Agency policy requires immediate supervisor notification and removal from client contact when impairment may create a safety risk.

What should the counselor do?

A) Use the agency's immediate safety process, report objective observations, and help prevent impaired client contact

B) Wait for group members to complain so the counselor has evidence of actual harm

C) Keep the concern private because reporting a colleague always violates professional loyalty

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Question 166

> A client has decided to reduce alcohol use at social events rather than pursue abstinence. The client identifies free drinks and lack of transportation as situations that often lead to drinking more than intended.

Which intervention best matches the client’s stated goal?

A) Require an abstinence commitment because moderation plans communicate approval of substance use.

B) Focus on why the client is unwilling to choose the goal most likely to eliminate risk.

C) Develop a concrete harm-reduction plan addressing drink limits, nonalcoholic alternatives, and transportation before the event.

Question 167

> Tariq has authorized a volunteer recovery mentor to participate in support planning. The mentor begins sending the counselor long daily messages about Tariq's routine, although Tariq originally requested only a weekly update about attendance at community activities.

How should the counselor manage the collaboration?

A) Reconfirm Tariq's desired involvement and establish a structured update schedule and relevant content with the mentor

B) Accept all daily messages because more collateral information always improves counseling decisions

C) End the mentor's involvement without consulting Tariq because the messages are too frequent

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Question 168

> A counselor has a telehealth appointment during an airport layover. The only available seating is public, and the network is open Wi-Fi. Practice policy requires a private setting and an approved secure connection for telehealth.

What should the counselor do?

A) Proceed with headphones because they prevent other people from hearing both sides

B) Proceed without video so nearby travelers cannot identify the client

C) Move to a compliant private setup or reschedule the appointment

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Question 169

> During migration to a new electronic health record, sample testing shows missing consent forms and altered service dates. Management wants to shut down the old system that evening to avoid another licensing fee.

What should the records lead recommend?

A) Decommission the old system because most clinical notes transferred successfully

B) Halt decommissioning, reconcile the discrepancies, and verify record completeness and integrity first

C) Ask counselors to recreate missing information from memory after the old system is erased

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Question 170

> A prospective member wants to join a closed 12-session coping-skills group but expects to miss the first four meetings because of travel. The group builds each week's exercises on material from earlier sessions, and an identical group will begin the following month.

What is the best intake decision?

A) Enroll the person now because attendance expectations should be discussed only after group begins

B) Explain the group's structure and attendance expectations and explore the later group or another suitable service

C) Deny all agency services because anticipated absences show resistance to counseling

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PART B — ANSWER KEY AND EXPLANATIONS

Answers correspond to the question numbers in Part A.

Question 1 — Correct: B

Correct Answer: Help the student identify documentation and access steps, practice making contact, and confirm a backup plan

Explanation: Practicing the access process supports the student's developmental move toward independent self-advocacy while still providing structure. Having a parent manage every step or waiting for failure would not prepare the student for the transition.

Question 2 — Correct: B

Correct Answer: Build portable notebook cues and child-owned coping steps, with caregiver supports that can be explained to a new placement

Explanation: Portable tools and child-owned skills can remain stable even if the caregiving setting changes. A plan dependent entirely on one household may not transfer, while postponement would withhold useful support during uncertainty.

Question 3 — Correct: A

Correct Answer: Apologize for the jargon, explain the idea in plain language with the client’s example, and check whether it makes sense.

Explanation: The counselor takes responsibility for accessible communication and verifies shared understanding without shaming the client. A glossary postpones repair and may preserve the same language barrier.

Question 4 — Correct: C

Correct Answer: Current conditions make the approach likely to cause harm or require abilities the client does not yet have

Explanation: A contraindication is based on a meaningful risk or prerequisite that affects suitability, not simply on anticipated effort. A request for pacing can often be incorporated through collaborative adaptation rather than treated as a reason to reject the approach.

Question 5 — Correct: C

Correct Answer: Complete the adult consent process, document the limited reminder role, and confirm the first adult-service appointment

Explanation: The stated policy requires new adult permissions, and the client's preference allows a specific, limited role for the parent. Automatically carrying forward youth permissions or excluding all family participation would override either policy or client choice.

Question 6 — Correct: C

Correct Answer: Bring the performance concern to the clinical supervisor and develop a monitored improvement plan, using consultation only as a supplement

Explanation: The omissions concern current clinical performance within an existing evaluative supervision relationship. The supervisor needs to help assess risk, observe improvement, and determine appropriate oversight; outside consultation may add expertise but cannot replace that responsibility.

Question 7 — Correct: A

Correct Answer: Obtain independent education on translation errors, cultural and linguistic bias, confidentiality, human review, and appropriate disclosure

Explanation: Machine-generated clinical translations can create meaning and access errors that fluency checks after use may not catch. Independent education prepares the clinic to evaluate limitations, review processes, privacy, and communication responsibilities before relying on the materials.

Question 8 — Correct: C

Correct Answer: Facilitate the program intake and maintain an agreed bridge until the receiving team confirms care

Explanation: Coordinated specialty care matches the client's clinical and educational needs, but a referral does not transfer responsibility until connection is confirmed. A brief bridge reduces the risk of losing care during intake.

Question 9 — Correct: A

Correct Answer: Review the plan in short sections, connect each step to a likely situation, and ask the client to explain how the steps would be used.

Explanation: Chunking, contextual examples, and client explanation turn written instructions into an actionable plan and reveal misunderstandings. The counselor should not independently replace a clinical discharge plan, and rereading alone does not address the client’s processing difficulty.

Question 10 — Correct: A

Correct Answer: The variables are related, but the correlation alone does not establish that one causes the other

Explanation: Correlation describes the direction and strength of association between variables. It does not establish causation or prove that the variables measure an identical construct.

Question 11 — Correct: C

Correct Answer: Apply the disclosed cancellation policy to the client without billing the insurer for a service not delivered

Explanation: The agreement permits a client fee, while the insurer expressly does not reimburse missed sessions. Submitting a therapy-service claim would falsely represent that counseling occurred, and billing both sources would compound the error.

Question 12 — Correct: B

Correct Answer: Examine the attraction to the role, obtain consultation, and decline the overlapping position while counseling is active

Explanation: The board role would place the counselor in decisions involving the active client, and the counselor's attraction to prestige may minimize that risk. Consultation and separation of the roles protect objectivity and the client's interests more effectively than limited abstention.

Question 13 — Correct: B

Correct Answer: Help the client plan a manageable form of opposite action, such as attending briefly and engaging with one supportive person.

Explanation: When an emotion-driven urge is not effective in the facts of the situation, a graded opposite action can interrupt avoidance and support a valued activity. Waiting for shame to vanish reinforces withdrawal, and calling the mistake a success is not credible emotional regulation.

Question 14 — Correct: A

Correct Answer: Follow the incident and downtime procedures immediately without attempting an unauthorized repair or payment

Explanation: Ransomware requires containment, evidence preservation, expert response, and continuity procedures. Continued use or personal payment could worsen the compromise, destroy evidence, or bypass organizational and legal obligations.

Question 15 — Correct: B

Correct Answer: “You have corrected me more than once, and I still got it wrong. I’m sorry. Please help me say it correctly now, and tell me how this has affected you.”

Explanation: The counselor accepts responsibility, takes an immediate corrective step, and makes room for the relational impact. Explaining that unfamiliar names are difficult shifts attention to the counselor and can minimize a repeated failure that matters to the client.

Question 16 — Correct: A

Correct Answer: Route the request through the required process and provide the accounting within the deadline

Explanation: The scenario gives the client a defined right and supplies the response period and scope. A treatment record describes care but does not substitute for a list of reportable disclosures.

Question 17 — Correct: C

Correct Answer: Describe the approval-seeking pattern and invite the member and group to explore what happens when the leader does not evaluate each contribution.

Explanation: The intervention uses the here-and-now pattern to reduce leader dependence and restore member-to-member engagement. Reassurance reinforces the pattern, while silent nonresponse without processing may feel punitive and offers little interpersonal learning.

Question 18 — Correct: B

Correct Answer: Reflective practice examines experience in a structured way to inform future action, whereas rumination repeats distress without useful movement

Explanation: Reflection links examination of experience to learning and a realistic next step. Rumination tends to recycle distress without generating new understanding or action, and the distinction does not depend on whether the process occurs alone.

Question 19 — Correct: B

Correct Answer: The description suggests depersonalization or derealization phenomena that require assessment of trauma, impairment, duration, and possible external causes

Explanation: Feeling detached or unreal while retaining awareness that the experience is subjective is consistent with dissociative phenomena rather than a fixed delusion. Further assessment must still address duration, impairment, trauma context, substances, and medical explanations.

Question 20 — Correct: C

Correct Answer: Score comparisons are more meaningful when the norm group is relevant to the population and purpose of use

Explanation: Norm-referenced interpretation depends on the comparison sample and the intended use of the measure. Sample size alone does not guarantee validity, and relevant client language and educational factors still require consideration.

Question 21 — Correct: C

Correct Answer: Treatment is producing meaningful functional gains, and the client should help decide whether the remaining plan needs revision

Explanation: The plan targeted sleep and participation as well as the experience of pain, and those functional outcomes have improved. Unchanged pain ratings should be discussed rather than treated as proof of total failure or ignored because other gains occurred.

Question 22 — Correct: C

Correct Answer: Help the student silence alerts, preserve necessary evidence with a chosen supportive adult, and decide how and when to access the accounts.

Explanation: This approach reduces repeated exposure, preserves options for reporting, and restores the student’s control with supportive involvement. Immediate deletion may destroy information and override choice, while continuous monitoring prolongs activation.

Question 23 — Correct: B

Correct Answer: Revoke proxy access promptly, document the change, and review any related privacy or safety concern

Explanation: The client who granted access has withdrawn it, and the system can implement that decision immediately. Delaying or requiring the spouse's agreement would expose new information contrary to the client's direction and could intensify a safety concern.

Question 24 — Correct: A

Correct Answer: Consider a sleep-transition phenomenon and continue monitoring context rather than diagnosing a psychotic disorder from this isolated experience

Explanation: A brief auditory experience limited to sleep onset with intact insight and no other psychotic features may be a hypnagogic phenomenon. The counselor should preserve diagnostic context rather than treating one isolated experience as sufficient for a psychotic or dissociative diagnosis.

Question 25 — Correct: C

Correct Answer: Blunted

Explanation: Blunted affect refers to a marked reduction in observable emotional expression. The client's reported internal pleasure does not make the outward affect euphoric, and no rapid shifts are described to suggest lability.

Question 26 — Correct: C

Correct Answer: Define the transportation problem precisely, brainstorm options without judging them, and evaluate one feasible next action.

Explanation: Structured problem solving targets the actual access barrier and moves from definition to options and action. Cognitive reframing alone does not create transportation, while renewed commitment to an unworkable route ignores the environmental constraint.

Question 27 — Correct: B

Correct Answer: Restating the essence in concise language and giving the client an opportunity to confirm or correct it.

Explanation: A paraphrase condenses the counselor’s understanding in fresh language and remains tentative enough for client correction. Verbatim repetition can sound mechanical, while adding an unexpressed motive turns paraphrase into unsupported interpretation.

Question 28 — Correct: C

Correct Answer: Confirm whether the client connected with the selected service and, with the client's involvement, address barriers or identify an alternative

Explanation: A closed-loop referral includes checking whether the intended connection actually occurred and responding when it did not. This approach keeps the client involved while preventing a referral from ending with contact information alone. Assuming that information transfer equals service access leaves barriers undiscovered.

Question 29 — Correct: C

Correct Answer: Locate a supported version or consult qualified assessment and cultural-language experts before using translated scores

Explanation: Clinical fluency does not establish competence in test translation or preserve validity through literal wording changes. A supported version or expert-guided process is needed before the scores can be interpreted as measuring the intended construct.

Question 30 — Correct: C

Correct Answer: Activate the required corrective process, protect clients and records, and make any mandated reports

Explanation: The supervisor has discovered fabricated services and billing, and policy specifies immediate containment and investigation. Private repayment or passive waiting would not correct records, address client impact, or fulfill possible reporting duties.

Question 31 — Correct: C

Correct Answer: Decline the undisclosed referral fee and base recommendations on client needs and service quality

Explanation: A hidden per-client payment creates a financial conflict that can distort referral judgment and client trust. Dividing the payment does not cure the undisclosed incentive; recommendations should rest on fit, quality, and client welfare.

Question 32 — Correct: C

Correct Answer: Replace the inaccurate text with current, individualized documentation and review whether similar errors entered prior notes

Explanation: Copy-forward tools do not relieve the counselor of responsibility for an accurate current record. Correcting the draft before signature and checking for earlier propagation protects clinical continuity and billing integrity.

Question 33 — Correct: B

Correct Answer: Engage affected people, define the barrier with relevant data, and identify feasible targets and partners for change

Explanation: Effective advocacy is grounded in the experiences of those affected and a clear understanding of the barrier and decision context. Premature solutions or public criticism can misidentify the problem and exclude useful partners.

Question 34 — Correct: A

Correct Answer: Help the worker settle physically, validate the distress, and identify an immediate supportive person or practical need.

Explanation: Early crisis support emphasizes stabilization, connection, and immediate needs rather than intensive cognitive or trauma processing. Even accurate factual challenge may be difficult to use while the worker is highly activated, and detailed retelling is not required.

Question 35 — Correct: C

Correct Answer: Clarify each person's goals and the counselor's role, then determine whether there is a mutually understood and suitable basis for conjoint work

Explanation: Incompatible unstated expectations can undermine consent and the suitability of conjoint counseling. The counselor should not impose reconciliation or defer role clarification until conflict develops.

Question 36 — Correct: C

Correct Answer: Ask the client what “my spirit feels crowded” means in the client’s own experience.

Explanation: Client language should be understood from the client’s frame of reference, especially when the counselor is uncertain about an idiom or metaphor. Assigning anxiety and overload may sound plausible but risks replacing the client’s meaning with the counselor’s assumption.

Question 37 — Correct: A

Correct Answer: Mentoring can support career growth and professional identity but does not replace required clinical supervision or establish competence by itself

Explanation: Mentors may provide perspective, encouragement, and career guidance without holding the formal oversight responsibilities of a clinical supervisor. Discussion with a mentor does not transfer accountability or independently demonstrate clinical competence.

Question 38 — Correct: B

Correct Answer: Use role standards, local outcome data, and stakeholder input to propose duties aligned with students' counseling needs

Explanation: Role standards and local evidence create a professionally grounded case for preserving counseling and prevention functions. Collaborative advocacy is more likely to improve the plan than unilateral refusal, while passive acceptance would disregard the documented student impact.

Question 39 — Correct: B

Correct Answer: Compare the systems and agree on consistent, developmentally appropriate symbols that support the child's goals across settings

Explanation: Coordinating the visual vocabulary can reduce frustration and reinforce the child's communication across services. The counselor and pathologist each contribute relevant expertise while keeping the child's goals central. Full counseling notes are not needed to compare and align the symbols.

Question 40 — Correct: A

Correct Answer: Evaluate trauma symptoms and alcohol use together, including their timing, interaction, impairment, and associated risks

Explanation: The alcohol use shows tolerance and occupational impact and may interact with trauma symptoms in a reinforcing pattern. Assessing both conditions together supports an accurate co-occurring formulation rather than treating either as ineligible for assessment until the other resolves.

Question 41 — Correct: B

Correct Answer: Include Mina in clarifying each provider's role, align the goals, and agree on how relevant updates will be exchanged

Explanation: Role clarification can reduce contradictory assignments while allowing each provider to contribute within an agreed scope. Mina's participation keeps the plan tied to her needs and preferences. Automatically ending one service is unnecessary when coordinated, complementary roles may be possible.

Question 42 — Correct: A

Correct Answer: Combine necessary structured questions with open-ended inquiry about the client's concerns, goals, strengths, and expectations

Explanation: Structured data can support completeness, but intake also requires the client's account and priorities. Abandoning all structure or moving immediately into intervention would leave important assessment information unaddressed.

Question 43 — Correct: B

Correct Answer: The score is one standard deviation below the mean

Explanation: A score of 85 is 15 points below a mean of 100, and the standard deviation is 15. This calculation does not identify an 85th-percentile rank.

Question 44 — Correct: C

Correct Answer: Disable or remove the device and verify a private environment before beginning the session

Explanation: An unapproved listening device creates a foreseeable confidentiality risk in the counselor's environment. General notice to the client would not substitute for the counselor's duty to establish a compliant private setting.

Question 45 — Correct: B

Correct Answer: Document the audiovisual limitation, record what was observable or reported, and avoid conclusions that required a clear image

Explanation: The note should represent both the available audio information and the limits of visual observation. Clear documentation of the technology limitation prevents later readers from treating unobserved features as assessed. One restricted channel does not require omitting all other mental-status information.

Question 46 — Correct: C

Correct Answer: Promptly assess fitness with appropriate medical and supervisory support and arrange coverage or workload changes when needed

Explanation: Slowed thinking and loss of the clinical narrative may already affect competent performance. The counselor should obtain appropriate assessment and oversight while protecting upcoming clients, rather than proceeding automatically or asking clients to evaluate impairment.

Question 47 — Correct: C

Correct Answer: Review current authoritative standards and guidance, use case-based education, and seek consultation on emerging questions

Explanation: Ethical knowledge changes and must be interpreted in new practice contexts. Current authoritative materials, applied education, and consultation provide a stronger learning process than relying on graduate training or informal local custom alone.

Question 48 — Correct: C

Correct Answer: Help the client organize the facts, feeling, and specific request, then rehearse the conversation with feedback.

Explanation: A structured interpersonal script plus rehearsal builds a middle path between avoidance and explosive expression. Written complaints may preserve avoidance, and relaxation without communication practice does not teach the requested skill.

Question 49 — Correct: A

Correct Answer: Certify only verifiable hours and address the discrepancy through the required training or board process

Explanation: Licensure verification must reflect supervision that actually occurred and can be supported by records. Clinical work performed without a supervision meeting does not become a supervision hour, and backdating notes would create a second misrepresentation.

Question 50 — Correct: A

Correct Answer: Examine the adaptation process, evidence, population limits, and relevant community perspectives before generalizing its use

Explanation: The phrase culturally adapted does not demonstrate how an intervention was developed, for whom it is supported, or whether affected communities informed it. Critical appraisal avoids both uncritical adoption and blanket rejection of culturally responsive adaptations.

Question 51 — Correct: C

Correct Answer: Protect the counseling content, clarify the counselor's role, and provide only information permitted by policy

Explanation: The principal's disciplinary purpose does not create an exception in the supplied facts. The counselor can clarify the role and offer permitted general information without turning confidential counseling content into disciplinary evidence.

Question 52 — Correct: A

Correct Answer: Arrange immediate medical and psychiatric evaluation because impaired intake and inability to engage safely indicate urgent risk

Explanation: Near-total refusal of food and fluids, physical weakness, and fixed poisoning beliefs create immediate medical and psychiatric concerns. Routine follow-up or delayed testing would not match the severity of impaired self-care.

Question 53 — Correct: B

Correct Answer: Providing understandable choices and supporting the client’s participation in decisions whenever capacity and safety permit.

Explanation: Empowerment preserves meaningful involvement and informed choice even when a crisis narrows available options. Automatically deciding for the client can unnecessarily remove control at a time when restoring agency may support stabilization.

Question 54 — Correct: B

Correct Answer: Each intervention is linked to an objective and supported by the case conceptualization

Explanation: Internal consistency means that the plan's actions have a clear rationale and are directed toward identified outcomes. Equal numbers of entries or identical deadlines may look orderly but do not demonstrate clinical alignment.

Question 55 — Correct: C

Correct Answer: Complete 10 additional qualifying non-self-study hours before attesting to compliance

Explanation: Only 10 of the 20 self-study hours may count, so the counselor currently has 20 qualifying hours rather than 30. Renewal attestation must be accurate at submission, not based on a promise to cure the shortage later.

Question 56 — Correct: C

Correct Answer: Prolonged grief disorder

Explanation: Persistent yearning, identity disruption, difficulty accepting the death, withdrawal, and marked impairment continue beyond 12 months and exceed cultural expectations. The duration and bereavement-specific pattern do not fit acute stress disorder or separation anxiety disorder.

Question 57 — Correct: C

Correct Answer: Clarify the financial arrangement and protect clinical information unless the client validly authorizes disclosure

Explanation: Payment by an outside organization does not automatically make its director part of the treatment team. The counselor may establish a clear funding agreement while keeping clinical information confidential unless the client authorizes a defined release.

Question 58 — Correct: A

Correct Answer: Make the caregiving pattern visible, validate the teen’s conflict, and help the family redistribute responsibilities in a developmentally appropriate way.

Explanation: The intervention addresses the family structure maintaining the teen’s exhaustion while acknowledging loyalty and guilt. Efficiency training leaves the excessive role intact, whereas unilateral refusal ignores the family’s practical needs and the value of collaborative change.

Question 59 — Correct: B

Correct Answer: Reschedule promptly with a qualified interpreter and confirm how the client can access support before then

Explanation: The client has declined to proceed without the communication access needed for meaningful participation, and there is no fact requiring an emergency exception. Prompt rescheduling and an interim access plan preserve continuity without relying on an unqualified family interpreter.

Question 60 — Correct: A

Correct Answer: Approved communication methods, privacy risks, expected response times, and procedures for urgent needs

Explanation: Informed consent for digital communication should establish how messages are sent, protected, monitored, and answered. It should also prevent the client from relying on a noncontinuous channel during an emergency.

Question 61 — Correct: A

Correct Answer: Help the client place the plan and key contacts where they can be reached quickly, then rehearse accessing and following the first step.

Explanation: Environmental access and behavioral rehearsal make the crisis plan more usable under cognitive strain. Memorizing the entire plan adds unnecessary burden, and a plan stored only in the record is unavailable when the client needs it.

Question 62 — Correct: A

Correct Answer: Revisit the agreement, help the partner plan prompt disclosure, and avoid continuing conjoint work while colluding with the secret

Explanation: The couple's informed agreement expressly rejects permanent individual secrecy for relationship-relevant information. The counselor should not secretly align with one partner or abruptly disclose without first using the agreed process to address the information and its effect on conjoint work.

Question 63 — Correct: A

Correct Answer: Record use and related effects for three weeks, then review the pattern and readiness for change

Explanation: The client is willing to observe consequences but has not chosen abstinence, making monitoring and collaborative review a realistic initial objective. Imposed abstinence exceeds current readiness, while avoiding the topic would fail to use the client's expressed willingness.

Question 64 — Correct: B

Correct Answer: Use the backup qualified interpreter service or reschedule according to the client's preference

Explanation: Effective language access is necessary for the client to participate meaningfully in the session. A backup qualified service or client-preferred rescheduling addresses the failed connection without pressuring the client to proceed in a less effective language. Public translation tools are not an adequate substitute for live interpretation of sensitive counseling content.

Question 65 — Correct: C

Correct Answer: Examine the conflict in consultation, evaluate ongoing clinical need, and make transition decisions based on client welfare

Explanation: The counselor's financial interest may be affecting termination decisions even though goals have been met. Consultation and a fresh evaluation of clinical need help separate business pressure from client welfare and informed transition planning.

Question 66 — Correct: B

Correct Answer: Review the discharge information with the client, coordinate authorized follow-up, and reconcile the outpatient plan

Explanation: Hospitalization has introduced new safety, medication, and follow-up information that should be integrated collaboratively. Simply resuming the old plan ignores important changes, while adopting every recommendation without discussion removes the client from planning.

Question 67 — Correct: A

Correct Answer: Assess current ability promptly and arrange appropriate coverage or rescheduling rather than beginning while significantly distracted

Explanation: The counselor's physical and cognitive response suggests a current inability to give a complex intake adequate attention. A brief fitness assessment and continuity plan protect the prospective client without requiring detailed personal disclosure.

Question 68 — Correct: A

Correct Answer: The typical spread of scores around the mean

Explanation: Standard deviation summarizes how dispersed scores are around the mean. It does not establish a cutoff's accuracy or indicate the percentage of items representing a construct.

Question 69 — Correct: B

Correct Answer: Briefly acknowledge the reaction, clarify that caring for the counselor is not the client’s responsibility, and explore the impact on the client.

Explanation: The counselor reestablishes the therapeutic roles and makes space for any burden, discomfort, or meaning created by the visible emotion. Accepting care reverses roles, while ignoring the event leaves the client alone with its impact.

Question 70 — Correct: C

Correct Answer: Briefly summarize the concerns and ask which one would be most useful to focus on today.

Explanation: A concise summary organizes the material and returns choice about session focus to the client. The counselor should not assume which issue deserves priority when the client can participate in selecting a workable focus.

Question 71 — Correct: B

Correct Answer: Refrain from independent clinical practice unless the required clinical license is obtained

Explanation: The state has expressly limited the school credential to school employment and excluded independent clinical practice. Changing the service title does not change the nature of psychotherapy or expand legal authority.

Question 72 — Correct: C

Correct Answer: Explain the service limitation, coordinate connection with the mobile team, and provide an appropriate bridge if feasible

Explanation: The counselor's available modalities do not match the client's current ability to access care, while a fitting service has openings. Transparent coordination and a bounded bridge support continuity better than making office attendance or unreliable public internet conditions of treatment.

Question 73 — Correct: A

Correct Answer: Explain the agency's actual eligibility policy, identify a reliable contact method with the client, and continue assessing service needs

Explanation: A permanent address is not an agency requirement, so the prior barrier should not be reproduced. Collaborative alternate contact planning permits intake to continue while respecting the client's security concerns.

Question 74 — Correct: B

Correct Answer: Observe the prohibition and, after it ends, retain the burden of showing that no exploitation or harm is likely

Explanation: Client initiation does not override the stated five-year prohibition. Passage of five years removes the categorical bar but does not create automatic approval; the counselor must still evaluate and document risks of exploitation or harm.

Question 75 — Correct: B

Correct Answer: Note that humor has appeared when caregiving resentment surfaces and invite each person to describe what happens next internally.

Explanation: The counselor tentatively identifies a repeated process and invites family members to explore its function and emotional consequences. Joining the joke would continue the avoidance, while silencing one member would personalize a family-wide pattern.

Question 76 — Correct: A

Correct Answer: Clarify the clients and roles, assess competence and conflicts, and obtain informed agreement about records and confidentiality

Explanation: Moving from individual to ongoing couples work changes who the clients are and how information, records, and roles will be handled. The client's request alone does not resolve conflicts or supply informed consent for the spouse.

Question 77 — Correct: C

Correct Answer: The consultation question, relevant case facts, recommendations received, and the counselor's resulting decision and follow-up

Explanation: These elements show why consultation occurred, what guidance was obtained, and how the counselor used it. A vague statement does not support later understanding of the decision. A full transcript would add unnecessary material without improving continuity.

Question 78 — Correct: C

Correct Answer: Withhold the export and verify the request with the director through the approved independent channel

Explanation: Realistic video can be fabricated or used in a compromised account, especially when the request is unusual and seeks bulk data. Independent confirmation through a known channel follows policy and prevents disclosure based on appearance alone.

Question 79 — Correct: B

Correct Answer: Verify the staff-assisted option and help Elena connect with it while allowing her to provide her own information

Explanation: The telephone option removes the digital barrier while preserving Elena's control over the information she supplies. Verifying the pathway before referral reduces another failed access attempt. Completing an application in her name would unnecessarily replace her participation.

Question 80 — Correct: B

Correct Answer: Document the access problem and help Caleb promptly contact the prescriber and pharmacy for verified continuity instructions

Explanation: The supply interruption requires timely guidance from the professionals responsible for prescribing and dispensing the medication. The counselor can document the barrier and facilitate that contact without changing how Caleb takes the medication. Advising skipped, reduced, or borrowed doses would exceed the counselor's role and could be unsafe.

Question 81 — Correct: C

Correct Answer: Explore the client's stated preference, explain available options and wait times, and arrange a suitable match when feasible

Explanation: The client has identified a personally meaningful privacy need, and the requested option is available. The counselor should respond to the individual preference without pathologizing it or generalizing it to everyone in the same religious community.

Question 82 — Correct: A

Correct Answer: Assess amounts, timing, withdrawal history, co-use risks, and current medical status, and facilitate prompt medical or substance-use evaluation

Explanation: Escalating benzodiazepine use, alcohol co-use, blackouts, and withdrawal symptoms create significant medical risk. Abrupt unsupervised cessation may be dangerous, while ignoring prescribed medication misuse would miss the central assessment concern.

Question 83 — Correct: A

Correct Answer: Explore a range of lower-risk options, such as documenting concerns, consulting a trusted ally, or making a limited request, and let the client choose.

Explanation: Assertiveness work should account for credible power and economic consequences rather than equate direct confrontation with progress. A range of graded options preserves agency and skill development without denying the workplace risk.

Question 84 — Correct: A

Correct Answer: Report the loss immediately and activate the required containment and assessment steps

Explanation: Encryption reduces risk but does not address the potentially active portal session or eliminate incident duties. Prompt session revocation, remote wipe, reporting, and assessment provide layered containment and an auditable response.

Question 85 — Correct: C

Correct Answer: Review the material orally in plain language, offer accessible alternatives, check understanding, and invite questions before proceeding

Explanation: A signature does not demonstrate meaningful understanding when the client cannot explain central terms. Adapting communication and using a comprehension check supports informed choice without transferring the decision to a family member.

Question 86 — Correct: A

Correct Answer: Acknowledge the impact, clarify the intention briefly, and ask what kind of pacing would feel more supportive.

Explanation: Therapeutic techniques are effective only as experienced in the particular relationship, so the counselor responds to impact and seeks collaborative adjustment. Defending silence as an established skill privileges theory over client feedback, while eliminating it forever is unnecessarily absolute.

Question 87 — Correct: A

Correct Answer: Add Imani's pet-reunification goal to the coordination plan and clarify the advocate's and nonprofit's related next steps

Explanation: Imani identified reunification as meaningful to settling safely into permanent housing. Including pet requirements and role-specific tasks in the plan addresses a concrete transition need before the foster arrangement ends. Immediate return would be premature until the building requirements are confirmed.

Question 88 — Correct: C

Correct Answer: Pause the intake and arrange a qualified interpreter who can communicate effectively in Rania's preferred dialect

Explanation: Credentials alone do not create effective communication when the client cannot understand the interpreter's dialect. Pausing and matching language support to Rania's stated need protects the accuracy of the intake. Expecting her to guess would shift responsibility for the communication failure to the client.

Question 89 — Correct: C

Correct Answer: Assess cognition, mood, daily functioning, medication and medical factors, and obtain relevant collateral information before forming a diagnosis

Explanation: Gradual cognitive change with functional decline warrants a broad assessment that includes possible neurocognitive, mood, medication, and medical contributors. Neither low mood nor one collateral report is sufficient by itself to establish the diagnosis.

Question 90 — Correct: A

Correct Answer: Review what produced symptom change and revise the plan to address the unresolved functional goal

Explanation: Lower symptom scores and worsening work functioning form a mixed result that should guide targeted revision. Declaring overall success or continuing unchanged would disregard the client's valued outcome and the functional data.

Question 91 — Correct: C

Correct Answer: Pair a supported return plan with alternate transportation and coping objectives while the school addresses harassment

Explanation: The school refusal is connected to a real environmental threat as well as anxiety, so the plan should address both. Requiring immediate use of the same bus could expose the student to continued harassment, while relaxation alone would omit the relevant context.

Question 92 — Correct: A

Correct Answer: Reduce services in stages, transfer specific tasks to chosen supports, and define indicators for renewed formal help

Explanation: Gradual de-intensification lets the family assume more ownership while preserving clearly defined support and reentry pathways. Abrupt withdrawal risks a gap, whereas unchanged service intensity disregards the family's gains and preference for greater independence.

Question 93 — Correct: C

Correct Answer: Counseling and research are separate choices, and the client may decline research without losing services

Explanation: The agency's policy makes research optional and independent of counseling eligibility. Separate consent prevents the intake setting from creating a false impression that access to care depends on research participation.

Question 94 — Correct: B

Correct Answer: Clarify item comprehension, consider whether the administration was valid, and integrate other interview and observational data before interpreting the score

Explanation: Misunderstood items can invalidate the meaning of the total score, so comprehension and administration conditions require review. The result should not override contradictory information, but one flawed administration also does not justify abandoning all formal assessment.

Question 95 — Correct: C

Correct Answer: Coordinate relevant medical care while targeting avoidance, coping, nutrition routines, and gradual return to valued activities

Explanation: The case includes interacting physical, behavioral, emotional, and social factors, all of which can inform the plan. Treating the symptoms as exclusively psychological or delaying counseling until all gastrointestinal symptoms resolve would oversimplify the formulation.

Question 96 — Correct: A

Correct Answer: Provide the copy through a secure available method and charge only any permitted reasonable cost

Explanation: The supplied rule makes a readily producible electronic format available to the requesting client. Requiring expensive paper or proprietary software would create an unsupported access barrier.

Question 97 — Correct: C

Correct Answer: Illness anxiety disorder

Explanation: The central feature is persistent fear of serious illness despite minimal symptoms and reassuring evaluations, accompanied by excessive checking and impairment. Somatic symptom disorder instead requires distressing somatic symptoms, while no deceptive behavior is described for factitious disorder.

Question 98 — Correct: C

Correct Answer: Develop the acceptable evidence-informed plan around scheduled draft submission and monitor whether it produces progress

Explanation: Both approaches are described as suitable, so the client's informed preference and measurable functional outcome can guide selection. Combining all techniques may dilute coherence, and counselor preference alone is not a sufficient reason to reject the client's choice.

Question 99 — Correct: B

Correct Answer: Document the revocation, stop future summaries, and explain that the prior authorized disclosure cannot be undone

Explanation: Under the stated policy, revocation controls future disclosures once received and documented. It does not retroactively invalidate a disclosure that was lawful when made, and the original expiration date no longer authorizes continued summaries.

Question 100 — Correct: C

Correct Answer: Review findings and recommendations relevant to Keisha's goal and note questions that require clarification

Explanation: Focused review helps the counselor prepare around Keisha's stated goal without treating a broad specialist report as an automatic counseling plan. Questions about technical findings can be identified for later clarification with Keisha or the evaluator. Ignoring the report would discard potentially useful functional information.

Question 101 — Correct: A

Correct Answer: The objective is realistically achievable from the client's baseline with available time and supports

Explanation: An attainable objective may still be challenging, but it is feasible in light of the client's starting point, resources, and time. It does not need to be effortless, and counselor preference cannot substitute for realistic client capacity.

Question 102 — Correct: B

Correct Answer: Predictive criterion validity

Explanation: Predictive criterion validity examines how well an earlier score forecasts a later relevant outcome. Internal consistency concerns relationships among test items, while face validity concerns whether a measure appears appropriate on its surface.

Question 103 — Correct: B

Correct Answer: Suspend destruction of the relevant records, preserve them securely, and document the legal hold

Explanation: A legal hold supersedes routine destruction for records potentially relevant to the claim. Secure preservation and documentation protect evidence integrity; transferring the records to the client would not end the practice's obligation.

Question 104 — Correct: A

Correct Answer: Interrupt the scapegoating, describe the group-wide pattern, and invite each member to examine a personal contribution to the tension.

Explanation: The leader protects the targeted member and redirects the group from collective blame toward shared process responsibility. Majority agreement does not make scapegoating therapeutically useful or establish that one person causes the entire group dynamic.

Question 105 — Correct: C

Correct Answer: The intern's status and role, the supervisor's involvement, relevant confidentiality implications, and available service options

Explanation: Training status, supervision, and service alternatives are material to the client's informed decision. Supervisor review does not remove the need to disclose who will provide the service and how supervision operates.

Question 106 — Correct: A

Correct Answer: Define the charging and morning routines, reinforce on-time days with agreed weekend recreation, and track arrival time

Explanation: The plan specifies antecedent routines, a client-valued reinforcer, and an observable outcome that can be reviewed. Unspecified punishment and occasional discussion do not create consistent contingencies or usable progress data.

Question 107 — Correct: A

Correct Answer: Designate a qualified records custodian and create secure procedures for notice, continuity, access, and financial obligations

Explanation: A professional will or continuity plan should authorize a qualified person to protect records, communicate with clients, manage appointments, and address unearned funds. Sharing personal passwords or ordering immediate destruction would undermine confidentiality, access, and retention duties.

Question 108 — Correct: A

Correct Answer: Evaluate the impact with supervision, use appropriate personal support, and consider rotating duties or reducing cumulative grief exposure

Explanation: Avoidance, guilt about positive experiences, and reduced emotional presence suggest cumulative grief exposure is affecting personal and professional functioning. A layered response can address the impact while preserving client care, whereas total social avoidance or permanent departure is premature.

Question 109 — Correct: B

Correct Answer: Explain the environmental limitation and facilitate care with the available qualified counselor

Explanation: The counselor cannot safely provide sessions in the available space, and a qualified accessible alternative is immediately available. Asking the client to forgo a required service animal or waiting for an allergic event would place the burden and risk on the client.

Question 110 — Correct: B

Correct Answer: Use an approved secure channel, establish response and emergency expectations, and verify intended recipients

Explanation: Secure technology alone is not sufficient; clients also need clear expectations about monitoring, response time, and emergencies. Personal texting and broad copying create avoidable privacy and boundary risks.

Question 111 — Correct: C

Correct Answer: Learn about economic access barriers and use client data to advocate for feasible scheduling or service alternatives

Explanation: The discontinuation pattern is linked to unpaid leave and limited hours, not simply individual time management. Combining contextual learning with client data supports targeted changes rather than equating identical schedules with equitable access.

Question 112 — Correct: B

Correct Answer: Verify the document's limits and obtain treatment and disclosure decisions from the competent client

Explanation: The document grants authority only over finances and property, not health-care consent or confidentiality. Because the client is competent, the client retains control of treatment and disclosure decisions.

Question 113 — Correct: A

Correct Answer: Follow the network agreement, correct any excess charges, and explain client responsibility accurately

Explanation: The contract defines the amounts the counselor may collect for covered services. A side agreement or a relabeled fee cannot be used to evade the plan's balance-billing restriction.

Question 114 — Correct: C

Correct Answer: Practice a balanced compassionate response that acknowledges the mistake and a corrective action, then compare its effect on motivation with the harsh response.

Explanation: The intervention addresses the client's concern empirically by combining accountability with compassion and observing motivational effects. Empty praise would not be credible, while avoiding the method leaves the belief that cruelty is necessary untested.

Question 115 — Correct: C

Correct Answer: Explain the instrument's limits, advocate for appropriate interpretation, and seek assessment consultation if needed

Explanation: Competence includes recognizing limits on how assessment results may be used, not only administering the tool correctly. A brief screener should inform broader evaluation rather than become an automatic discharge rule, and altering scores would invalidate the process.

Question 116 — Correct: B

Correct Answer: Explore each partner's meanings of trust, privacy, and safety, then facilitate a mutually consented boundary rather than prescribe one.

Explanation: The counselor helps the couple examine needs and negotiate consent without declaring a universal rule for intimacy. Prescribing password sharing could validate coercion, while technological separation alone would not resolve the underlying meanings.

Question 117 — Correct: A

Correct Answer: Collaborate on realistic sleep, activity, and session objectives that fit the client's stated observance and preferences

Explanation: The client has identified a temporary, meaningful context that changes what is feasible. Adapting objectives preserves treatment direction without pathologizing observance or pretending that scheduling constraints are irrelevant.

Question 118 — Correct: A

Correct Answer: Seek supervision promptly, reinforce professional boundaries, and arrange a careful transfer if objectivity cannot be maintained

Explanation: Changes in dress and unnecessary message review show that the attraction is beginning to influence professional behavior. Early supervision and firm boundaries may protect care, with transfer considered if the counselor cannot maintain objectivity; disclosing the attraction would burden the client.

Question 119 — Correct: A

Correct Answer: Ask which immediate concern matters most, address one step at a time, and check the client's understanding before adding information.

Explanation: Acute stress can reduce information processing, so prioritization, small chunks, and comprehension checks make support usable. Reading everything aloud preserves the overload, while withholding all information removes access to resources the client may need now.

Question 120 — Correct: B

Correct Answer: Document the client's communication preferences, explain relevant privacy limitations, and use an available safer contact method

Explanation: Intake should establish a contact method that reflects the client's privacy circumstances and the clinic's actual capabilities. A uniform voicemail rule would create avoidable disclosure risk, while eliminating reminders without collaboration may create another access barrier.

Question 121 — Correct: B

Correct Answer: Explore how the client wants the approaches to complement each other and coordinate only with the client’s informed permission.

Explanation: This response respects the client’s cultural resources, autonomy, and control over information sharing. Contacting the healer without permission violates the client’s stated preference, while requiring a choice between approaches imposes an unnecessary division.

Question 122 — Correct: C

Correct Answer: Help the couple rehearse a time-limited pause that includes a clear statement of when they will return to the conversation.

Explanation: A structured pause addresses physiological flooding without turning withdrawal into indefinite avoidance or abandonment. A stated return time gives the pursuing partner predictability while both partners practice interrupting the cycle.

Question 123 — Correct: B

Correct Answer: Learn from qualified community and professional sources, examine the counselor's assumptions, and help the agency design feasible access options

Explanation: The attendance pattern has credible structural explanations that the counselor initially misread as low commitment. Community-informed learning and service adaptation address both the assumption and the practical barriers without treating occupational similarity as a prerequisite for competence.

Question 124 — Correct: A

Correct Answer: Verify the clinic's current access information and help Jamal reconnect promptly with the medication team

Explanation: The missed appointment resulted from an outdated access pathway, so current information and prompt reconnection are the relevant supports. The medication team can determine any clinical response to the delay. Recommending a substitute would place the counselor in a prescribing role.

Question 125 — Correct: A

Correct Answer: Recognize the misuse of positional power, stop the sarcastic responses, and use supervision to restore respectful and consistent facilitation

Explanation: Mandated participation increases the importance of using authority carefully rather than humiliating a member for dissent. The counselor must address the personal reaction and facilitation behavior; coercion or removal based only on objections would intensify the power problem.

Question 126 — Correct: B

Correct Answer: Maintain confidentiality and require a valid basis before disclosing any protected information

Explanation: A completed unlawful act without an ongoing threat does not create a confidentiality exception under the stated facts. Even confirming client status can reveal protected information, so an informal police inquiry is not enough.

Question 127 — Correct: B

Correct Answer: Learn about the practical barriers with participants and advocate for scheduling, child-care, or delivery options that address them

Explanation: Participant reports identify concrete access barriers rather than lack of interest. Staff learning and feasible program changes respond to those conditions, while stricter messaging or exclusion would preserve the barrier.

Question 128 — Correct: B

Correct Answer: Avoidant/restrictive food intake disorder

Explanation: The restriction is driven by sensory characteristics and has caused weight loss and nutritional deficiency without weight-shape concerns. That pattern supports avoidant/restrictive food intake disorder rather than anorexia nervosa.

Question 129 — Correct: A

Correct Answer: Develop shift-specific sleep routines and a feasible anchor period while continuing to monitor sleep outcomes

Explanation: The fixed schedule conflicts with an employment condition the client cannot change, despite consistent effort with other strategies. A feasible adaptation allows the counselor to test outcomes without labeling a structural barrier as nonadherence.

Question 130 — Correct: A

Correct Answer: Verify the client's identity and obtain the client's valid consent before beginning telehealth services

Explanation: A competent adult's partner cannot supply consent merely because the service may be beneficial. Identity verification and the client's own valid agreement are required before relying on the electronic form.

Question 131 — Correct: C

Correct Answer: Link the early cue to a rehearsed pause, identify support after the meeting, and define when to seek follow-up

Explanation: The plan builds on a personally recognized warning sign and a skill the client has already used successfully. Indefinite avoidance prevents practice of gains, while motivation without a specific response leaves the known trigger insufficiently addressed.

Question 132 — Correct: C

Correct Answer: Flag the missing item, plan to clarify it with the client, and interpret the measure only as allowed after clarification

Explanation: The scoring instructions explicitly rule out substituting zero for the missing response. Preparing to clarify the item preserves the measure's intended use and avoids creating a misleading score. The incomplete form may still become usable after the client addresses the omission.

Question 133 — Correct: B

Correct Answer: Limit or suspend affected duties, arrange client coverage, and resume only when competent performance is restored

Explanation: The documented cognitive effects are already impairing professional performance, regardless of the medication's lawful prescription. Protecting clients requires limiting affected work and arranging continuity until competence is restored.

Question 134 — Correct: B

Correct Answer: Establish transparent, relevant criteria and apply and document them consistently

Explanation: A sliding-fee program can allow flexibility without basing financial access on personal preference. Written, relevant criteria and consistent documentation reduce unfair treatment while preserving reduced-fee services.

Question 135 — Correct: A

Correct Answer: Record that Rowan completed the reflection in the agreed audio format, summarize the relevant themes and response, and state the next plan

Explanation: The audio reflection fulfilled the adapted assignment and produced material relevant to the counseling goal. The note should document the completed task, clinical response, and next step rather than labeling a format difference as noncompletion. Unrelated recording content need not be copied into the record.

Question 136 — Correct: B

Correct Answer: Record the counselor's breathing instruction under the intervention and the client's practice and reported change under the response

Explanation: Separating the counselor's action from the client's response makes the note clinically interpretable. The original sentence describes only what the client did and experienced, leaving the intervention unclear. The plan section should identify future action rather than replace documentation of what occurred.

Question 137 — Correct: B

Correct Answer: Specific post-treatment supports, responsibilities, and contingencies for maintaining gains

Explanation: Aftercare is future-focused and identifies how gains will be supported after active treatment ends. A historical reconstruction or a vague invitation to contact any provider does not assign concrete actions or contingencies.

Question 138 — Correct: C

Correct Answer: Client-defined goals, decisions needed, participant responsibilities, and a method and date for follow-up

Explanation: This agenda links the meeting to the client's aims and converts discussion into accountable next steps. Decisions, responsibilities, and follow-up make coordination observable after the meeting. Professional seniority is not a substitute for a task-focused plan.

Question 139 — Correct: B

Correct Answer: When a focused prompt can help balance participation and briefly hear from each willing member.

Explanation: A round provides structure and broad access to participation around a defined question while allowing members to pass. It is not designed to manufacture agreement or create an extended individual session within the group.

Question 140 — Correct: A

Correct Answer: Respond within the disclosed service window and reinforce the messaging expectations if needed

Explanation: The message is nonurgent and falls within the portal terms that the client received. An after-hours timestamp does not transform it into an emergency or justify removing access; the counselor should follow the stated response window.

Question 141 — Correct: C

Correct Answer: Provide an appropriate adult referral and, with permission, coordinate only the caregiver support relevant to the child's plan

Explanation: The parent's needs warrant their own provider rather than being treated within the child's sessions. Limited, authorized coordination can still support implementation of the child's home plan without making the parent a second client in that treatment.

Question 142 — Correct: B

Correct Answer: To identify times the problem is absent or less intense and examine resources associated with those times.

Explanation: Exception questions help clients notice existing strengths, contextual differences, and behaviors linked to better moments. They are not primarily tools for testing honesty or tracing a problem to its origin.

Question 143 — Correct: C

Correct Answer: Map a recent episode step by step to identify triggers, early body cues, actions, and points for a different response.

Explanation: A detailed behavior chain slows the sequence and identifies earlier intervention points that the client currently misses. Thought suppression is unlikely to teach regulation, and consequences alone do not reveal where a different response can be inserted.

Question 144 — Correct: C

Correct Answer: Arrange qualified interpretation in the client's preferred language rather than assuming conversational fluency is sufficient

Explanation: Everyday conversation does not establish that a client can fully understand or express complex counseling content in English. The client's request is direct information about what support is needed for meaningful participation. A relative is not an equivalent substitute for qualified interpretation.

Question 145 — Correct: A

Correct Answer: Compare the course with credible competency standards, evidence, instructor qualifications, and opportunities to demonstrate skill

Explanation: A certificate label does not establish that training meets meaningful professional standards. Evaluating instruction, evidence, assessed practice, and recognition helps the counselor distinguish a marketing title from genuine competence development.

Question 146 — Correct: A

Correct Answer: Promptly use supervision and recovery supports to assess functioning and arrange transfer if competent care cannot be maintained

Explanation: The emerging cravings and concentration difficulty require prompt evaluation of both counselor wellness and client care. Supervision and recovery supports may permit safe continuation, but transfer is appropriate if functioning cannot be stabilized; client disclosure would shift responsibility inappropriately.

Question 147 — Correct: B

Correct Answer: Coordinate appropriate medical input and use suitable plan components while clarifying which exercises are safe

Explanation: Deliberately provoking physical sensations warrants clarification because the client's seizure disorder may affect exercise selection. The counselor need not assume either that every standard exercise is safe or that all behavioral treatment is permanently contraindicated.

Question 148 — Correct: C

Correct Answer: Explore what the information would mean to the client, explain the privacy limit, and discuss safer ways to address the need for understanding.

Explanation: The counselor maintains personal and family privacy while treating the request as meaningful rather than simply rejecting it. Exploring the underlying need can lead to appropriate ways of building cultural trust without disclosing identifying details.

Question 149 — Correct: C

Correct Answer: Lower the pace, reflect the urgent need behind the anger, and offer a choice between reviewing an appeal step or identifying another resource.

Explanation: This response validates the core concern, reduces interactional intensity, and offers limited productive choices. A detailed policy explanation is hard to process during high arousal, while an immediate ultimatum may intensify the power struggle when no threat is present.

Question 150 — Correct: B

Correct Answer: Avoid confirming client status, preserve the information according to applicable practice procedures, and discuss collateral involvement with the client if the intake occurs

Explanation: An unsolicited message does not authorize confirmation that the person is a client or permit disclosure to the sender. The counselor can handle the received information appropriately and later seek the client's perspective and permission regarding collateral involvement.

Question 151 — Correct: B

Correct Answer: Document the gain and collaboratively add an interview-preparation objective while maintaining useful application habits

Explanation: Meeting an objective early is information for progression, not a reason to retain a completed task or close the broader goal. The next objective should address the newly identified barrier while preserving behaviors that remain useful.

Question 152 — Correct: A

Correct Answer: Explore the client’s concern, explain how reminder information is handled, and let the client choose an acceptable reminder method.

Explanation: The counselor links the present reaction to the client’s stated context, provides transparent information, and preserves choice among available methods. Continuing texts as involuntary exposure dismisses the surveillance history, while disabling everything without consultation also removes agency.

Question 153 — Correct: A

Correct Answer: Remove the quotation marks and identify the content as the client's reported meaning

Explanation: Quotation marks indicate exact language, which the counselor knows was not recorded. Identifying the sentence as a paraphrased client report preserves its accurate meaning without overstating precision. Useful reported content does not need to be deleted merely because it is not a direct quotation.

Question 154 — Correct: A

Correct Answer: Compare schedule alternatives with the client and authorized stakeholders while limiting communication to the participation issue

Explanation: The authorization supports focused logistical coordination that can preserve two activities the client values. Clinical details are not needed to compare schedules and identify workable accommodations. Requiring the client to abandon the team would bypass collaborative problem-solving.

Question 155 — Correct: B

Correct Answer: Comparing scores assigned independently by two trained observers watching the same behavior

Explanation: Interrater reliability concerns agreement between evaluators observing the same target. Repeating a measure over time assesses test-retest reliability, while correlation with another construct measure addresses validity evidence.

Question 156 — Correct: A

Correct Answer: To communicate attention and invite the client to continue without directing the content.

Explanation: Minimal encouragers convey presence while leaving the client in control of what comes next. They do not necessarily signal agreement and should not covertly steer the client toward the counselor's preferred topic.

Question 157 — Correct: B

Correct Answer: Review the confidentiality agreement and its safety limits with both, explain how updates can be handled, and invite questions before proceeding.

Explanation: Clear orientation creates realistic expectations and supports trust for both adolescent and parent under the stated agency arrangement. Promising total secrecy is inaccurate, while removing the parent before explaining the framework can intensify confusion and conflict.

Question 158 — Correct: A

Correct Answer: Hoarding disorder

Explanation: Persistent difficulty discarding, distress associated with discarding, and clutter that compromises living areas are defining features of hoarding disorder under the supplied exclusions. A personality style or situational fear does not account for the accumulation and blocked living space.

Question 159 — Correct: A

Correct Answer: Review progress and remaining needs, create a focused ending plan, and offer affordable continuation or reentry options

Explanation: The client has the right to end care, and the financial constraint can be addressed through a planned, clinically informed transition. Coercing completion or ending without review would fail to support maintenance of partial gains.

Question 160 — Correct: C

Correct Answer: Verify appropriate coverage, authorization, and scope compliance for the volunteer setting

Explanation: A license establishes professional authority only within applicable scope and does not itself provide insurance for every setting. The counselor should confirm both the shelter's requirements and independent coverage before beginning clinical work.

Question 161 — Correct: B

Correct Answer: Evaluate the client's ability to understand information, appreciate its personal relevance, reason about options, and communicate a stable choice

Explanation: Capacity is decision specific and cannot be inferred solely from a psychiatric diagnosis or disagreement with a recommendation. The assessment should examine the functional abilities required for the actual decision.

Question 162 — Correct: B

Correct Answer: Examine the personal meaning of lateness, consult as needed, repair the inconsistency, and apply policy based on clinical facts

Explanation: The counselor's history and satisfaction at the client's distress show that a personal reaction is shaping a harsher limit. Reflection, repair, and consistent application restore fairness while still allowing attendance to be addressed clinically.

Question 163 — Correct: A

Correct Answer: Explain the required focus and confidentiality limits, then link alcohol-related review with the student's academic goal

Explanation: The counselor should be transparent about the mandated elements while using the flexibility allowed by the program to build client-relevant goals. Ignoring the required focus or excluding the student's academic concern would weaken informed collaboration.

Question 164 — Correct: A

Correct Answer: With Andre's input, align counseling times with the dialysis schedule and establish a continuity plan for treatment-related disruptions

Explanation: The attendance pattern has a predictable treatment-related context that can be addressed through coordinated scheduling. A continuity plan respects Andre's wish to remain in counseling while preparing for disruptions. The dialysis social worker can collaborate but should not decide Andre's emotional readiness.

Question 165 — Correct: A

Correct Answer: Use the agency's immediate safety process, report objective observations, and help prevent impaired client contact

Explanation: The observed signs and imminent group session create the safety risk described by agency policy. Waiting for actual harm would leave clients exposed, while a factual report avoids speculation about the colleague's diagnosis.

Question 166 — Correct: C

Correct Answer: Develop a concrete harm-reduction plan addressing drink limits, nonalcoholic alternatives, and transportation before the event.

Explanation: The client has chosen a reduction goal and identified predictable risk conditions, so a specific plan can lower harm and support self-monitoring. Imposing abstinence would override the stated goal and may reduce engagement.

Question 167 — Correct: A

Correct Answer: Reconfirm Tariq's desired involvement and establish a structured update schedule and relevant content with the mentor

Explanation: Tariq should continue to define how the mentor participates in his support plan. A clear schedule and content boundary can preserve useful collaboration without allowing excessive collateral material to drive counseling. Ending the relationship unilaterally would also disregard Tariq's choice.

Question 168 — Correct: C

Correct Answer: Move to a compliant private setup or reschedule the appointment

Explanation: Both the public setting and open network fail the practice's stated safeguards. Headphones or audio-only communication cannot protect what the counselor says, the screen, or the network transmission.

Question 169 — Correct: B

Correct Answer: Halt decommissioning, reconcile the discrepancies, and verify record completeness and integrity first

Explanation: Missing forms and changed dates show that the migration has not preserved reliable records. Deleting the source system before reconciliation could make errors permanent and impair care, access, and compliance.

Question 170 — Correct: B

Correct Answer: Explain the group's structure and attendance expectations and explore the later group or another suitable service

Explanation: The cumulative closed-group format makes missing the first third of meetings clinically and practically relevant to suitability. Transparent discussion and an available later group address the mismatch without treating travel as resistance or denying all services.
